SHUBHAM
CANCER CENTRE
Near Sola Over Bridge, Hebatpur Road,
Opp. Zydus Hospital, Thaltej,
Ahmedabad 380 059, (Gujarat).
Mobile No.: +91 95123 40431

CANCER ASSOCIATES
CONSULTANT CANCER SURGEONS

ADDRESSOGRAPH
If ADDRESSOGRAPH not Available, Write
PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

ADMISSION CONSENT

Admission Ward & Bed No: _____

AUTHORIZATION FOR INVESTIGATIONS, PROCEDURES, TREATMENT, RELEASE OF INFORMATION AND PAYMENT

(The contents of this form have been explained to me in language I understand)
(ફોર્મમાં રજૂ કરવામાં દર્શાવેલ વિગતો મને મારી માતૃભાષામાં સમજવામાં આવી છે.)

I, the undersigned, do hereby agree and give my consent / for my _____ (relation & name of patient) for admission to _____ Hospital and I hereby authorize the hospital physicians, nurses & other staff including medical, nursing and paramedical students/trainee of the hospital to provide such care and administer such diagnostic, radiological and / or therapeutic procedures and treatment as, in the judgement of the above physician(s) is deemed necessary or advisable in my / the above patient's care / care of my newborn (in the case of obstetrical patients). This includes all routine diagnostic tests and procedures, including diagnostic x-rays, the administration and / or injection of pharmaceutical products and medications and withdrawal of blood for laboratory examinations. I acknowledge the fact that the hospital has the authority to dispose off specimens taken for laboratory or pathology examination. In addition, I hereby authorize any person caring for me to review and / or release my personal health information to other healthcare providers treating me during this hospitalization. I certify that I have read and understand this form and that no guarantees have been made to me as to the results of treatment or examinations done in the hospital.

I also hereby authorize and direct _____ Hospital and / or its representatives having treated me to release to government agencies, insurance carriers, or others who are financially liable for my hospitalization and medical care, all information needed to substantiate payment for such hospitalization and medical care and to permit representatives thereof to examine and make copies of all records relating to such care and treatment.

I have been explained about the approximate cost of treatment / package. I have also been explained that the cost of medical treatment depends on the number of days of hospitalization, area of hospitalization, investigations performed, drugs and consumables used, procedures and surgeries performed, professional fee charged, nature of illness and variety of other factors. Therefore, the estimate given to me is only a rough indication of the approximate costs towards hospitalization. The final bill may, therefore, vary significantly from the estimate, and the amount mentioned in the final bill shall be the final amount payable to the hospital. I also understand that interim intimation of the bill will be presented on a regular basis. Payment shall be made by me by cash / credit / DD.

I understand that for credit cases (TPA covered or corporate patients), authorization letter is essential at the time of admission. In case the authorization letter is not available, patient shall be treated as cash patient and advance deposits shall have to be made. Once authorization is obtained, the amount paid as advance shall be refunded to me / patient and if there is no authorization till discharge, the bill shall be settled in full by me / patient. It has been explained that a certain fixed amount will be required to be paid at the time of admission. In case of a package, the entire amount needs to be paid on admission. I undertake full responsibility of clearing all dues payable to the hospital authorities during my / patient's stay here in case of any eventuality occurs. I promise to arrange the full payment of dues either by me or immediate legal heirs of the patient immediately. I understand that in case of my failure to clear off all dues to the hospital, at the time of my / patient's discharge, the hospital authorities may take civil or criminal action against me.

I acknowledge the receipt of the IP Guide of the hospital, which includes Patient's Rights and Responsibilities and I agree to abide by them. All cash, jewellery and other valuables shall be removed by me to place of safety. I shall not hold the hospital authorities responsible for any kind of loss sustained by me or my family.

Signature of Patient / Attendant (Relationship) : _____ Date & Time : _____

Sig nature of Front Office Personal : _____ Date &Time: _____

Witness Name & Signature : _____ Date &Time: _____

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ADMISSION FORM

Please (✓) tick whichever is applicable.

This form is to be filled up by patient's relatives in his/her hand writting.

Time Admission :
Date :

(A) Patient Details

Name of Patient : _____ Age : _____

Name of the Relative : _____ Date of Birth : _____

Name of Ref. Dr. : _____ Marital Status : Married / Unmarried

Name of Attending Dr. : _____ Gender : [ ] Male [ ] Female

(B) Permanent Address

Address : _____

_____

City : _____ Pin code : _____ Dist : _____ State : _____

(C) Patient Contact No.

STD Code : _____ (R) _____ (0) _____ (M) _____

E-mail _____

Contact No. of Relative

(1) Name : _____ Contact No. _____ Relation with patient _____

(2) Name : _____ Contact No. _____ Relation with patient _____

(E.) Consent

I the undersigned _____ am aware of all the rules and regulations of the hospital. | have been given all the information regarding the class of room I have selected. I understand the doctors visit fee, operative charges (if applicable) and the surcharges applicable according to the class of room selected. I have discussed with the consultant regarding the surgical and visit fee. I have also understood that incase of up gradation of class, the charges will be counted from the day of admission. I have received satisfactory answer to all my questions and also agree to pay the deposit as per hospital rules.

Selected Room No. : _____

Is patient diabetic ? ☐ Yes ☐ No ☐ Don't know

Patient's Name : _____

Relative's Name : _____ Relation with Patient : _____ Sign : _____

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દાખલ થતી વખતનું સંમતિ પત્રક

પ્રવેશવોર્ડનં : .....

તપાસ,પ્રોસીજર, સારવાર, માહિતીઓ આપવા માટેરકમ ચુકવણી માટેની પરવાનગી.

(ફોર્મમાં રજૂ કરવામાં દર્શાવેલ વિગતો મને મારી માતૃભાષામાં સમજવામાં આવી છે.)

હું ..... / મારા દર્દીને આ હોસ્પિટલમાં દાખલ માટે કબુલ થાઉં છું તથા પરવાનગી આપું છું. આ સાથે આ હોસ્પિટલ અને એના બધા મેડિકલ , નર્સિંગ , હાઉસ સ્ટાફ અને મેડિકલ, નર્સિંગ, પેરામેડિકલ સ્ટુડન્ટ / ટ્રેઇની અને હોસ્પિટલના બધા જ કર્મચારીઓને વિનંતી કરું છું તેમજ પરવાનગી આપું છું, કે ઉપર જણાવેલા મારા દર્દીનેજરૂરિયાત હોય તે પ્રમાણે બધી જ સારવાર, તાપસ, એક્સ-રે કરવામાં આવે જે ડોક્ટરોએ ઓર્ડર કરેલી હોય તેમની સમજ પ્રમાણે. આની અંદર બધીજ પ્રોસિજર, તાપસ, એક્સ-રે, ઈન્જેકશન, દવાઓ, તાપસ માટે લોહી લેવું. હું જાણું છું કે હોસ્પિટલને સત્તા છે કે તે લેબોરેટરી માટે લીધેલા ખાસ અથવા પેથોલોજી તાપસ માટે લીધેલા સ્પેસિમેન તેમને ઠીક લાગે તેપ્રમાણે તેનો નિકાલ કરી શકે છે.

હું અહીં, સરકારશ્રી, ઈન્સ્યોરન્સ કંપનીના પ્રતિનિધિ અથવા જેઓ મારી આર્થિક જવાબદારી ધરાવે છે તેમને મારી સારવાર સંબંધિત બધી જ માહિતી આપવા માટે આ હોસ્પિટલ અથવા તેમના પ્રતિનિધિને સંમતિ અને નિર્દેશ આપું છું જેથી મારી સારવારની આર્થિક ચૂકવણી કરવામાં સરળતા રહે તથા તેની દસ્તાવેજી નોંધ રાખી શકાય.

હું મારી તબિયત વિષેની બધી માહિતીઓ જો બીજા કોઈ ક્લીનીક , હોસ્પિટલ કે સંરથાને આપવી પડે તો મારી સંભાળ લેતી વ્યક્તિઓને તેની પરવાનગી આપું છું જેથી મારી સારવાર કે તપારાના પરિણામો વિષે કોઈ ખાતરી આપવામાં નથી આવી.

મને સારવારના પેકેજની કિંમત સમજાવવામાં આવી છે તેમજ જાણકારી આપવામાં આવી છે કે એ કિંમત હોસ્પિટલમાં જે ધ્વિસો માટે રહેવું પડે, જે વોર્ડમાં રહેવાનું પસંદ કરીએ, તપાસ, ધ્વાઓ અને બીજી વસ્તુઓ ઉપયોગમાં લેવામાં આવે તેની ઉપર અવલંબે છે. તેથી મને જે કિંમત કહેવામાં આવી છે તેમાં ફેરફાર પણ થઈ શકે છે અને તેથી છેલ્લા બિલની રકમ જુદી હોઈ શકે છે. રોજના ટેમ્પરરી બિલ કરતા, રકમની ચુકવણી રોકડા / ક્રેડિટ / ડિમાન્ડ ડ્રાફ્ટથી થઈ શકે છે. મને ખબર છે કે જો ક્રેડિટ (કંપનીના દર્દીઓ / ટી. પી. એ.) હોય તો પરવાનગી પત્ર કંપનીનો હોવો જરૂરી છે. દાખલ થતી વખતે જો પત્ર મળી શકે તેમ ન હોય તો દર્દને રોકડા પૈસા ભરવા પડશે. પરવાનગી પત્ર મળતાજે રકમપાછી દેવામાં આવશે અને જો રજા મળતા સુધી પરવાનગી પત્ર નહીં મળે તો પૂરેપૂરું બિલ મારે/ દર્દીએ ભરી દેવુંપડશે. મનેએ પણ સમજાવવામાં આવ્યું છેકે એડમિશન વખતે એક ચોક્કસ રકમ ભરવાની થાય છે. જો પેકેજ હોય તો, પુરી રકમ એડમિશન વખતેજ ભરવાપાત્ર છે. હું/ દર્દી, બધું જ બાડી રહેલું બિલ હોસ્પિટલને ભરપાઈ કરવાની જવાબદારી લઉં છું, જો કંઈ અજુગતું બને તો જો હું / દર્દી અથવા કાયદેસરના જવાબદાર સબંધી બિલ ભરપાઈ ન કરીએ તો હોસ્પિટલના સત્તાવાળાઓ મારી સામે સિવિલ / ક્રિમિનલ કાયદેસરની કાર્યવાહી કરવાનો અધિકાર ધરાવે છે.

મને દાખલ થયેલા દર્દીઓ માતેલી ગાઈડ મળી છે. જેમાં દર્દીના હક્કો તથા જવાબદારીઓ સમજાવવામાં આવ્યા છે એન્ડ હું તે રીતે વર્તવા કબુલ થાઉં છું. મારી પાસે જે રોકડા, ઘરેણાં કે બીજી કિંમતી વસ્તુઓ છે તે હું નિજી સલામત જગ્યાએ મૂકી દઈશ અનેહું મને કે મારા કુટુંબને કંઈ પણ નુકશાન થશે તો હોસ્પિટલને જવાબદાર નહીં ગણું.

દર્દી ની સહી / નજીકના સગાનું નામ ..... તારીખ / સમય : .....

દાખલ કરનારા ઓફિસરનું નામ, સહી અને આઈ ડી ..... તારીખ / સમય : .....

સાક્ષી નું નામ અને સહી : ..... તારીખ / સમય : .....

ન્યાયક્ષેત્રે અમદાવાદ ને આધીન રહેશે.

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એડમીશન ફોમ

જયાં લાગુ પડતું હોય ત્યાં (✓) કરવું.

આ ફોર્મ દર્દી / દર્દીના સગાએ પોતાના અક્ષરોમાં ભરવું જરૂરી છે. આપેલ માહિતીમાં પાછળથી કોઈ ફેરફાર થઈ શકશે નહીં.

એડમીશન તારીખ :
સમય:

(A) દર્દીની માહિતી

દર્દીનું નામ : _____ ઉંમર: _____

દર્દી સાથે આવેલ સંબંધીનું નામ : _____ જન્મ તારીખ : _____

રેફ. ડૉક્ટરનું નામ : _____ વૈવાહિક સ્થિતિ: પરિણીત / અપરિણીત

કન્સલ્ટન્ટ ડૉક્ટરનું નામ : _____ જાતિ : પુરુષ ☐ સ્ત્રી ☐

(B) કાયમી પત્રવ્યવહાર માટે સરનામું

સરનામું : _____

_____

શહેર : _____ પીન કોડ : _____ જીલ્લો : _____ રાજ્ય : _____

(C) દર્દીના સંપર્ક નંબર

એસ.ટી.ડી. કોડ: _____ રહેઠાણ : _____ ઓફીસ: _____ મો : _____ _____

ઈ-મેલ : _____

-સાથે જે સંબંધી હોય તેનો સંપર્ક નંબર

૧. નામ : _____ સંપર્ક નંબર : _____ દર્દી સાથે સંબંધ : _____

2. નામ : _____ સંપર્ક નંબર : _____ દર્દી સાથે સંબંધ : _____

(E) સંમતિ

હું નીચે સહી કરનાર _____ હોસ્પિટલના તમામ નીતિ-નિયમોથી વાકેફ છું. મેં પસંદ કરેલા કલાસના રૂમ ચાર્જની સંપુર્ણ માહિતી મને આપવામાં આવેલ છે. ડૉકટર વીઝીટ ફી, ઓપરેટીવ ચાર્જીસ (જો હોય તો) અને કલાસ પ્રમાણેના સરચાર્જની માહિતી હું સમજેલ છું. મેં કન્સલ્ટન્ટ સાથે સર્જીકલ તેમજ વીઝીટ ફી ની પણ ચર્ચા કરેલ છે. કલાસ અપગ્રેડેશનમાં ચાર્જીસ એડમીશનની તારીખથી ગણવામાં આવશે તે પણ મને સમજાવવામાં આવેલ છે. મારા તમામ પ્રશ્નોના સંતોષકારક જવાબ મને મળ્યા છે તથા હું હોસ્પિટલના નિયમો મુજબ ડિપોઝીટ ભરવા સંમત છું.

પસંદગીનો રૂમનં.. _____

દર્દીને ડાયાબીટીસ છે. ? હા ના ખબરનથી

દર્દીનું નામ :.....

દર્દીના સગાનું નામ:.....દર્દી સાથેનો સંબંધ :.....સહી .....

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Ahmedabad 380 059, (Gujarat).
Mobile No.: +91 95123 40431

CANCER ASSOCIATES
CONSULTANT CANCER SURGEONS

AH/MRD/FORM - 29 Page no : 1

ADDRESSOGRAPH
If ADDRESSOGRAPH not Available, Write
PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

TREATMENT SHEET

Date :_____

Allergic to_____ Time :_____

DIET :	Day of Stay :	Duty	Nurse Name	Doctor Name	Incharge Nurse
		M			
	Post OP. Day :	E			
		N			

Diagnosis :

Surgery :

02 Theraphy : Nasal Prongs ☐ Mask ☐

Flow_____L/min.

Star Time : _____

Stop Time :_____

Sr No.		Route	Dosage & Timing					

Last 24 Hours INPUT | OUTPUT Total		
INTAKE	OUTPUT	
IV :	U/O :	Drain Output
Oral :	RTA :	Drain - 1
RTF / NJG :		Drain - 2
FJ :		Drain - 3
Infusion :	STOOL :	lieostomy
	Vomiting :	Colostomy :
Total Intake :	Total Output :	
Balance		

RBS: | | | | |

INSULIN : | | | | |

AG : | | | | |

Consultant's Signature

Continue...

Sr No.		Route	Dosage & Timing					

IMPORTANT EVENTS & TODAY'S PLAN

References :

1 _____

2 _____

3 _____

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CANCER CENTRE
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CANCER ASSOCIATES
CONSULTANT CANCER SURGEONS

AH/MRD/FORM - 61 Page no : 1

ADDRESSOGRAPH
If ADDRESSOGRAPH not Available, Write
PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

INTAKE AND OUTPUT CHART

Weight: _____ **KgDiet:** _____ **Date:** _____

Time	INTAKE						OUTPUT					
	ORAL/RT FEEDING (ml)						RT Aspiration	Urine	Vomit	Stool	Drain -1	Drain -2
	TYPE	AMOUNT	IV1	IV2	IV3	IV4	(in ml)	(in ml)	(in ml)	(No. of Times)	(in ml)	(in ml)
08:00												
09:00												
10:00												
11:00												
12:00												
13:00												
14:00												
15:00												
16:00												
17:00												
18:00												
19:00												
20:00												
Total												
21:00												
22:00												
23:00												
00:00												
01:00												
02:00												
03:00												
04:00												
05:00												
06:00												
07:00												
Total												

Total Intake:	Total Output:	Balance:	Previous Day Balance:	Remarks

Name : **Sign :**

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CANCER ASSOCIATES
CONSULTANT CANCER SURGEONS

ADDRESSOGRAPH
If ADDRESSOGRAPH not Available, Write
PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

DAILY VITAL SINGS CHART

Time	Temp	Pulse	Resp	BP	Oxygen Status	SPO 2	AG	Pain Score	Fall Risk Score	GCS Score	EWS	Sign of Nurse
08:00	°F	/min	/min	mmHg	lit/min	%	cm					
09:00	°F	/min	/min	mmHg	lit/min	%	cm					
10:00	°F	/min	/min	mmHg	lit/min	%	cm					
11:00	°F	/min	/min	mmHg	lit/min	%	cm					
12:00	°F	/min	/min	mmHg	lit/min	%	cm					
13:00	°F	/min	/min	mmHg	lit/min	%	cm					
14:00	°F	/min	/min	mmHg	lit/min	%	cm					
15:00	°F	/min	/min	mmHg	lit/min	%	cm					
16:00	°F	/min	/min	mmHg	lit/min	%	cm					
17:00	°F	/min	/min	mmHg	lit/min	%	cm					
18:00	°F	/min	/min	mmHg	lit/min	%	cm					
19:00	°F	/min	/min	mmHg	lit/min	%	cm					
20:00	°F	/min	/min	mmHg	lit/min	%	cm					
21:00	°F	/min	/min	mmHg	lit/min	%	cm					
22:00	°F	/min	/min	mmHg	lit/min	%	cm					
23:00	°F	/min	/min	mmHg	lit/min	%	cm					
00:00	°F	/min	/min	mmHg	lit/min	%	cm					
01:00	°F	/min	/min	mmHg	lit/min	%	cm					
02:00	°F	/min	/min	mmHg	lit/min	%	cm					
03:00	°F	/min	/min	mmHg	lit/min	%	cm					
04:00	°F	/min	/min	mmHg	lit/min	%	cm					
05:00	°F	/min	/min	mmHg	lit/min	%	cm					
06:00	°F	/min	/min	mmHg	lit/min	%	cm					
07:00	°F	/min	/min	mmHg	lit/min	%	cm					

Name : _____ **Sign :** _____

Date : _____ **Time :** _____

24 HOURS NURSE'S OVER SHEET

Date : _____

	Morning	Evening	Night
Introduction			
Situation			
Background			
Assessment			
Recommendation			

Name & ID of Nurse :	(Morning Duty)	Name & ID of Nurse :	(Evening Duty)	Name & ID of Nurse :	(Night Duty)
	(Evening Duty)		(Night Duty)		(Morning Duty)

Name & Sign of Sister Incharge : _____ Time : _____ Date : _____

ISBAR : Introduction - Identify yourself who you are talking lo and who you ar talking about. Situation What Is the current siuation concems observations, EWS eic ? Background - What Is the relevant background ? This helps to set the scene to interpret the situation above accurately

Assessment - What do you think the problem is ? This requires the interpretation of the situation and background information to make an educated conclusion about what is going on. **Recommendation -** What do you need them to do? What do you recemmend should be done to come the current situation ?

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CANCER ASSOCIATES
CONSULTANT CANCER SURGEONS

AH/MRD/FORM - 72

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PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

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CANCER ASSOCIATES
CONSULTANT CANCER SURGEONS

AH/MRD/FORM - 29 Page no : 1

ADDRESSOGRAPH
If ADDRESSOGRAPH not Available, Write
PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

TREATMENT SHEET

Date :_____

Allergic to_____ Time :_____

DIET :	Day of Stay :	Duty	Nurse Name	Doctor Name	Incharge Nurse
		M			
	Post OP. Day :	E			
		N			

Diagnosis :

Surgery :

02 Theraphy : Nasal Prongs ☐ Mask ☐

Flow_____L/min.

Star Time : _____

Stop Time :_____

Sr No.		Route	Dosage & Timing					

Last 24 Hours INPUT | OUTPUT Total		
INTAKE	OUTPUT	
IV :	U/O :	Drain Output
Oral :	RTA :	Drain - 1
RTF / NJG :		Drain - 2
FJ :		Drain - 3
Infusion :	STOOL :	lieostomy
	Vomiting :	Colostomy :
Total Intake :	Total Output :	
Balance		

RBS: | | | | |

INSULIN : | | | | |

AG : | | | | |

Consultant's Signature

Continue...

Sr No.		Route	Dosage & Timing					

IMPORTANT EVENTS & TODAY'S PLAN

References :

1 _____

2 _____

3 _____

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CANCER ASSOCIATES
CONSULTANT CANCER SURGEONS

AH/MRD/FORM - 61 Page no : 1

ADDRESSOGRAPH
If ADDRESSOGRAPH not Available, Write
PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

INTAKE AND OUTPUT CHART

Weight: _____ **KgDiet:** _____ **Date:** _____

Time	INTAKE						OUTPUT					
	ORAL/RT FEEDING (ml)						RT Aspiration	Urine	Vomit	Stool	Drain -1	Drain -2
	TYPE	AMOUNT	IV1	IV2	IV3	IV4	(in ml)	(in ml)	(in ml)	(No. of Times)	(in ml)	(in ml)
08:00												
09:00												
10:00												
11:00												
12:00												
13:00												
14:00												
15:00												
16:00												
17:00												
18:00												
19:00												
20:00												
Total												
21:00												
22:00												
23:00												
00:00												
01:00												
02:00												
03:00												
04:00												
05:00												
06:00												
07:00												
Total												

Total Intake:	Total Output:	Balance:	Previous Day Balance:	Remarks

Name : **Sign :**

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CANCER ASSOCIATES
CONSULTANT CANCER SURGEONS

ADDRESSOGRAPH
If ADDRESSOGRAPH not Available, Write
PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

DAILY VITAL SINGS CHART

Time	Temp	Pulse	Resp	BP	Oxygen Status	SPO 2	AG	Pain Score	Fall Risk Score	GCS Score	EWS	Sign of Nurse
08:00	°F	/min	/min	mmHg	lit/min	%	cm					
09:00	°F	/min	/min	mmHg	lit/min	%	cm					
10:00	°F	/min	/min	mmHg	lit/min	%	cm					
11:00	°F	/min	/min	mmHg	lit/min	%	cm					
12:00	°F	/min	/min	mmHg	lit/min	%	cm					
13:00	°F	/min	/min	mmHg	lit/min	%	cm					
14:00	°F	/min	/min	mmHg	lit/min	%	cm					
15:00	°F	/min	/min	mmHg	lit/min	%	cm					
16:00	°F	/min	/min	mmHg	lit/min	%	cm					
17:00	°F	/min	/min	mmHg	lit/min	%	cm					
18:00	°F	/min	/min	mmHg	lit/min	%	cm					
19:00	°F	/min	/min	mmHg	lit/min	%	cm					
20:00	°F	/min	/min	mmHg	lit/min	%	cm					
21:00	°F	/min	/min	mmHg	lit/min	%	cm					
22:00	°F	/min	/min	mmHg	lit/min	%	cm					
23:00	°F	/min	/min	mmHg	lit/min	%	cm					
00:00	°F	/min	/min	mmHg	lit/min	%	cm					
01:00	°F	/min	/min	mmHg	lit/min	%	cm					
02:00	°F	/min	/min	mmHg	lit/min	%	cm					
03:00	°F	/min	/min	mmHg	lit/min	%	cm					
04:00	°F	/min	/min	mmHg	lit/min	%	cm					
05:00	°F	/min	/min	mmHg	lit/min	%	cm					
06:00	°F	/min	/min	mmHg	lit/min	%	cm					
07:00	°F	/min	/min	mmHg	lit/min	%	cm					

Name : _____ **Sign :** _____

Date : _____ **Time :** _____

24 HOURS NURSE'S OVER SHEET

Date : _____

	Morning	Evening	Night
Introduction			
Situation			
Background			
Assessment			
Recommendation			

Name & ID of Nurse :	(Morning Duty)	Name & ID of Nurse :	(Evening Duty)	Name & ID of Nurse :	(Night Duty)
	(Evening Duty)		(Night Duty)		(Morning Duty)

Name & Sign of Sister Incharge : _____ Time : _____ Date : _____

ISBAR : Introduction - Identify yourself who you are talking lo and who you ar talking about. Situation What Is the current siuation concems observations, EWS eic ? Background - What Is the relevant background ? This helps to set the scene to interpret the situation above accurately

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CANCER ASSOCIATES
CONSULTANT CANCER SURGEONS

AH/MRD/FORM - 72

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PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

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CONSULTANT CANCER SURGEONS

AH/MRD/FORM - 29 Page no : 1

ADDRESSOGRAPH
If ADDRESSOGRAPH not Available, Write
PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

TREATMENT SHEET

Date :_____

Allergic to_____ Time :_____

DIET :	Day of Stay :	Duty	Nurse Name	Doctor Name	Incharge Nurse
		M			
	Post OP. Day :	E			
		N			

Diagnosis :

Surgery :

02 Theraphy : Nasal Prongs ☐ Mask ☐

Flow_____L/min.

Star Time : _____

Stop Time :_____

Sr No.		Route	Dosage & Timing					

Last 24 Hours INPUT | OUTPUT Total		
INTAKE	OUTPUT	
IV :	U/O :	Drain Output
Oral :	RTA :	Drain - 1
RTF / NJG :		Drain - 2
FJ :		Drain - 3
Infusion :	STOOL :	lieostomy
	Vomiting :	Colostomy :
Total Intake :	Total Output :	
Balance		

RBS: | | | | |

INSULIN : | | | | |

AG : | | | | |

Consultant's Signature

Continue...

Sr No.		Route	Dosage & Timing					

IMPORTANT EVENTS & TODAY'S PLAN

References :

1 _____

2 _____

3 _____

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CONSULTANT CANCER SURGEONS

AH/MRD/FORM - 61 Page no : 1

ADDRESSOGRAPH
If ADDRESSOGRAPH not Available, Write
PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

INTAKE AND OUTPUT CHART

Weight: _____ **KgDiet:** _____ **Date:** _____

Time	INTAKE						OUTPUT					
	ORAL/RT FEEDING (ml)						RT Aspiration	Urine	Vomit	Stool	Drain -1	Drain -2
	TYPE	AMOUNT	IV1	IV2	IV3	IV4	(in ml)	(in ml)	(in ml)	(No. of Times)	(in ml)	(in ml)
08:00												
09:00												
10:00												
11:00												
12:00												
13:00												
14:00												
15:00												
16:00												
17:00												
18:00												
19:00												
20:00												
Total												
21:00												
22:00												
23:00												
00:00												
01:00												
02:00												
03:00												
04:00												
05:00												
06:00												
07:00												
Total												

Total Intake:	Total Output:	Balance:	Previous Day Balance:	Remarks

Name : **Sign :**

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CANCER ASSOCIATES
CONSULTANT CANCER SURGEONS

ADDRESSOGRAPH
If ADDRESSOGRAPH not Available, Write
PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

DAILY VITAL SINGS CHART

Time	Temp	Pulse	Resp	BP	Oxygen Status	SPO 2	AG	Pain Score	Fall Risk Score	GCS Score	EWS	Sign of Nurse
08:00	°F	/min	/min	mmHg	lit/min	%	cm					
09:00	°F	/min	/min	mmHg	lit/min	%	cm					
10:00	°F	/min	/min	mmHg	lit/min	%	cm					
11:00	°F	/min	/min	mmHg	lit/min	%	cm					
12:00	°F	/min	/min	mmHg	lit/min	%	cm					
13:00	°F	/min	/min	mmHg	lit/min	%	cm					
14:00	°F	/min	/min	mmHg	lit/min	%	cm					
15:00	°F	/min	/min	mmHg	lit/min	%	cm					
16:00	°F	/min	/min	mmHg	lit/min	%	cm					
17:00	°F	/min	/min	mmHg	lit/min	%	cm					
18:00	°F	/min	/min	mmHg	lit/min	%	cm					
19:00	°F	/min	/min	mmHg	lit/min	%	cm					
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22:00	°F	/min	/min	mmHg	lit/min	%	cm					
23:00	°F	/min	/min	mmHg	lit/min	%	cm					
00:00	°F	/min	/min	mmHg	lit/min	%	cm					
01:00	°F	/min	/min	mmHg	lit/min	%	cm					
02:00	°F	/min	/min	mmHg	lit/min	%	cm					
03:00	°F	/min	/min	mmHg	lit/min	%	cm					
04:00	°F	/min	/min	mmHg	lit/min	%	cm					
05:00	°F	/min	/min	mmHg	lit/min	%	cm					
06:00	°F	/min	/min	mmHg	lit/min	%	cm					
07:00	°F	/min	/min	mmHg	lit/min	%	cm					

Name : _____ **Sign :** _____

Date : _____ **Time :** _____

24 HOURS NURSE'S OVER SHEET

Date : _____

	Morning	Evening	Night
Introduction			
Situation			
Background			
Assessment			
Recommendation			

Name & ID of Nurse :	(Morning Duty)	Name & ID of Nurse :	(Evening Duty)	Name & ID of Nurse :	(Night Duty)
	(Evening Duty)		(Night Duty)		(Morning Duty)

Name & Sign of Sister Incharge : _____ Time : _____ Date : _____

ISBAR : Introduction - Identify yourself who you are talking lo and who you ar talking about. Situation What Is the current siuation concems observations, EWS eic ? Background - What Is the relevant background ? This helps to set the scene to interpret the situation above accurately

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CANCER ASSOCIATES
CONSULTANT CANCER SURGEONS

AH/MRD/FORM - 72

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AH/MRD/FORM - 29 Page no : 1

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If ADDRESSOGRAPH not Available, Write
PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

TREATMENT SHEET

Date :_____

Allergic to_____ Time :_____

DIET :	Day of Stay :	Duty	Nurse Name	Doctor Name	Incharge Nurse
		M			
	Post OP. Day :	E			
		N			

Diagnosis :

Surgery :

02 Theraphy : Nasal Prongs ☐ Mask ☐

Flow_____L/min.

Star Time : _____

Stop Time :_____

Sr No.		Route	Dosage & Timing					

Last 24 Hours INPUT | OUTPUT Total		
INTAKE	OUTPUT	
IV :	U/O :	Drain Output
Oral :	RTA :	Drain - 1
RTF / NJG :		Drain - 2
FJ :		Drain - 3
Infusion :	STOOL :	lieostomy
	Vomiting :	Colostomy :
Total Intake :	Total Output :	
Balance		

RBS: | | | | |

INSULIN : | | | | |

AG : | | | | |

Consultant's Signature

Continue...

Sr No.		Route	Dosage & Timing					

IMPORTANT EVENTS & TODAY'S PLAN

References :

1 _____

2 _____

3 _____

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AH/MRD/FORM - 61 Page no : 1

ADDRESSOGRAPH
If ADDRESSOGRAPH not Available, Write
PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

INTAKE AND OUTPUT CHART

Weight: _____ **KgDiet:** _____ **Date:** _____

Time	INTAKE						OUTPUT					
	ORAL/RT FEEDING (ml)						RT Aspiration	Urine	Vomit	Stool	Drain -1	Drain -2
	TYPE	AMOUNT	IV1	IV2	IV3	IV4	(in ml)	(in ml)	(in ml)	(No. of Times)	(in ml)	(in ml)
08:00												
09:00												
10:00												
11:00												
12:00												
13:00												
14:00												
15:00												
16:00												
17:00												
18:00												
19:00												
20:00												
Total												
21:00												
22:00												
23:00												
00:00												
01:00												
02:00												
03:00												
04:00												
05:00												
06:00												
07:00												
Total												

Total Intake:	Total Output:	Balance:	Previous Day Balance:	Remarks

Name : **Sign :**

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ADDRESSOGRAPH
If ADDRESSOGRAPH not Available, Write
PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

DAILY VITAL SINGS CHART

Time	Temp	Pulse	Resp	BP	Oxygen Status	SPO 2	AG	Pain Score	Fall Risk Score	GCS Score	EWS	Sign of Nurse
08:00	°F	/min	/min	mmHg	lit/min	%	cm					
09:00	°F	/min	/min	mmHg	lit/min	%	cm					
10:00	°F	/min	/min	mmHg	lit/min	%	cm					
11:00	°F	/min	/min	mmHg	lit/min	%	cm					
12:00	°F	/min	/min	mmHg	lit/min	%	cm					
13:00	°F	/min	/min	mmHg	lit/min	%	cm					
14:00	°F	/min	/min	mmHg	lit/min	%	cm					
15:00	°F	/min	/min	mmHg	lit/min	%	cm					
16:00	°F	/min	/min	mmHg	lit/min	%	cm					
17:00	°F	/min	/min	mmHg	lit/min	%	cm					
18:00	°F	/min	/min	mmHg	lit/min	%	cm					
19:00	°F	/min	/min	mmHg	lit/min	%	cm					
20:00	°F	/min	/min	mmHg	lit/min	%	cm					
21:00	°F	/min	/min	mmHg	lit/min	%	cm					
22:00	°F	/min	/min	mmHg	lit/min	%	cm					
23:00	°F	/min	/min	mmHg	lit/min	%	cm					
00:00	°F	/min	/min	mmHg	lit/min	%	cm					
01:00	°F	/min	/min	mmHg	lit/min	%	cm					
02:00	°F	/min	/min	mmHg	lit/min	%	cm					
03:00	°F	/min	/min	mmHg	lit/min	%	cm					
04:00	°F	/min	/min	mmHg	lit/min	%	cm					
05:00	°F	/min	/min	mmHg	lit/min	%	cm					
06:00	°F	/min	/min	mmHg	lit/min	%	cm					
07:00	°F	/min	/min	mmHg	lit/min	%	cm					

Name : _____ **Sign :** _____

Date : _____ **Time :** _____

24 HOURS NURSE'S OVER SHEET

Date : _____

	Morning	Evening	Night
Introduction			
Situation			
Background			
Assessment			
Recommendation			

Name & ID of Nurse :	(Morning Duty)	Name & ID of Nurse :	(Evening Duty)	Name & ID of Nurse :	(Night Duty)
	(Evening Duty)		(Night Duty)		(Morning Duty)

Name & Sign of Sister Incharge : _____ Time : _____ Date : _____

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CANCER ASSOCIATES
CONSULTANT CANCER SURGEONS

AH/MRD/FORM - 72

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AH/MRD/FORM - 29 Page no : 1

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TREATMENT SHEET

Date :_____

Allergic to_____ Time :_____

DIET :	Day of Stay :	Duty	Nurse Name	Doctor Name	Incharge Nurse
		M			
	Post OP. Day :	E			
		N			

Diagnosis :

Surgery :

02 Theraphy : Nasal Prongs ☐ Mask ☐

Flow_____L/min.

Star Time : _____

Stop Time :_____

Sr No.		Route	Dosage & Timing					

Last 24 Hours INPUT | OUTPUT Total		
INTAKE	OUTPUT	
IV :	U/O :	Drain Output
Oral :	RTA :	Drain - 1
RTF / NJG :		Drain - 2
FJ :		Drain - 3
Infusion :	STOOL :	lieostomy
	Vomiting :	Colostomy :
Total Intake :	Total Output :	
Balance		

RBS: | | | | |

INSULIN : | | | | |

AG : | | | | |

Consultant's Signature

Continue...

Sr No.		Route	Dosage & Timing					

IMPORTANT EVENTS & TODAY'S PLAN

References :

1 _____

2 _____

3 _____

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AH/MRD/FORM - 61 Page no : 1

ADDRESSOGRAPH
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INTAKE AND OUTPUT CHART

Weight: _____ **KgDiet:** _____ **Date:** _____

Time	INTAKE						OUTPUT					
	ORAL/RT FEEDING (ml)						RT Aspiration	Urine	Vomit	Stool	Drain -1	Drain -2
	TYPE	AMOUNT	IV1	IV2	IV3	IV4	(in ml)	(in ml)	(in ml)	(No. of Times)	(in ml)	(in ml)
08:00												
09:00												
10:00												
11:00												
12:00												
13:00												
14:00												
15:00												
16:00												
17:00												
18:00												
19:00												
20:00												
Total												
21:00												
22:00												
23:00												
00:00												
01:00												
02:00												
03:00												
04:00												
05:00												
06:00												
07:00												
Total												

Total Intake:	Total Output:	Balance:	Previous Day Balance:	Remarks

Name : **Sign :**

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CONSULTANT CANCER SURGEONS

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DAILY VITAL SINGS CHART

Time	Temp	Pulse	Resp	BP	Oxygen Status	SPO 2	AG	Pain Score	Fall Risk Score	GCS Score	EWS	Sign of Nurse
08:00	°F	/min	/min	mmHg	lit/min	%	cm					
09:00	°F	/min	/min	mmHg	lit/min	%	cm					
10:00	°F	/min	/min	mmHg	lit/min	%	cm					
11:00	°F	/min	/min	mmHg	lit/min	%	cm					
12:00	°F	/min	/min	mmHg	lit/min	%	cm					
13:00	°F	/min	/min	mmHg	lit/min	%	cm					
14:00	°F	/min	/min	mmHg	lit/min	%	cm					
15:00	°F	/min	/min	mmHg	lit/min	%	cm					
16:00	°F	/min	/min	mmHg	lit/min	%	cm					
17:00	°F	/min	/min	mmHg	lit/min	%	cm					
18:00	°F	/min	/min	mmHg	lit/min	%	cm					
19:00	°F	/min	/min	mmHg	lit/min	%	cm					
20:00	°F	/min	/min	mmHg	lit/min	%	cm					
21:00	°F	/min	/min	mmHg	lit/min	%	cm					
22:00	°F	/min	/min	mmHg	lit/min	%	cm					
23:00	°F	/min	/min	mmHg	lit/min	%	cm					
00:00	°F	/min	/min	mmHg	lit/min	%	cm					
01:00	°F	/min	/min	mmHg	lit/min	%	cm					
02:00	°F	/min	/min	mmHg	lit/min	%	cm					
03:00	°F	/min	/min	mmHg	lit/min	%	cm					
04:00	°F	/min	/min	mmHg	lit/min	%	cm					
05:00	°F	/min	/min	mmHg	lit/min	%	cm					
06:00	°F	/min	/min	mmHg	lit/min	%	cm					
07:00	°F	/min	/min	mmHg	lit/min	%	cm					

Name : _____ **Sign :** _____

Date : _____ **Time :** _____

24 HOURS NURSE'S OVER SHEET

Date : _____

	Morning	Evening	Night
Introduction			
Situation			
Background			
Assessment			
Recommendation			

Name & ID of Nurse :	(Morning Duty)	Name & ID of Nurse :	(Evening Duty)	Name & ID of Nurse :	(Night Duty)
	(Evening Duty)		(Night Duty)		(Morning Duty)

Name & Sign of Sister Incharge : _____ Time : _____ Date : _____

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AH/MRD/FORM - 72

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AH/MRD/FORM - 29 Page no : 1

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TREATMENT SHEET

Date :_____

Allergic to_____ Time :_____

DIET :	Day of Stay :	Duty	Nurse Name	Doctor Name	Incharge Nurse
		M			
	Post OP. Day :	E			
		N			

Diagnosis :

Surgery :

02 Theraphy : Nasal Prongs ☐ Mask ☐

Flow_____L/min.

Star Time : _____

Stop Time :_____

Sr No.		Route	Dosage & Timing					

Last 24 Hours INPUT | OUTPUT Total		
INTAKE	OUTPUT	
IV :	U/O :	Drain Output
Oral :	RTA :	Drain - 1
RTF / NJG :		Drain - 2
FJ :		Drain - 3
Infusion :	STOOL :	lieostomy
	Vomiting :	Colostomy :
Total Intake :	Total Output :	
Balance		

RBS: | | | | |

INSULIN : | | | | |

AG : | | | | |

Consultant's Signature

Continue...

Sr No.		Route	Dosage & Timing					

IMPORTANT EVENTS & TODAY'S PLAN

References :

1 _____

2 _____

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AH/MRD/FORM - 61 Page no : 1

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INTAKE AND OUTPUT CHART

Weight: _____ **KgDiet:** _____ **Date:** _____

Time	INTAKE						OUTPUT					
	ORAL/RT FEEDING (ml)						RT Aspiration	Urine	Vomit	Stool	Drain -1	Drain -2
	TYPE	AMOUNT	IV1	IV2	IV3	IV4	(in ml)	(in ml)	(in ml)	(No. of Times)	(in ml)	(in ml)
08:00												
09:00												
10:00												
11:00												
12:00												
13:00												
14:00												
15:00												
16:00												
17:00												
18:00												
19:00												
20:00												
Total												
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23:00												
00:00												
01:00												
02:00												
03:00												
04:00												
05:00												
06:00												
07:00												
Total												

Total Intake:	Total Output:	Balance:	Previous Day Balance:	Remarks

Name : **Sign :**

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Time	Temp	Pulse	Resp	BP	Oxygen Status	SPO 2	AG	Pain Score	Fall Risk Score	GCS Score	EWS	Sign of Nurse
08:00	°F	/min	/min	mmHg	lit/min	%	cm					
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10:00	°F	/min	/min	mmHg	lit/min	%	cm					
11:00	°F	/min	/min	mmHg	lit/min	%	cm					
12:00	°F	/min	/min	mmHg	lit/min	%	cm					
13:00	°F	/min	/min	mmHg	lit/min	%	cm					
14:00	°F	/min	/min	mmHg	lit/min	%	cm					
15:00	°F	/min	/min	mmHg	lit/min	%	cm					
16:00	°F	/min	/min	mmHg	lit/min	%	cm					
17:00	°F	/min	/min	mmHg	lit/min	%	cm					
18:00	°F	/min	/min	mmHg	lit/min	%	cm					
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00:00	°F	/min	/min	mmHg	lit/min	%	cm					
01:00	°F	/min	/min	mmHg	lit/min	%	cm					
02:00	°F	/min	/min	mmHg	lit/min	%	cm					
03:00	°F	/min	/min	mmHg	lit/min	%	cm					
04:00	°F	/min	/min	mmHg	lit/min	%	cm					
05:00	°F	/min	/min	mmHg	lit/min	%	cm					
06:00	°F	/min	/min	mmHg	lit/min	%	cm					
07:00	°F	/min	/min	mmHg	lit/min	%	cm					

Name : _____ **Sign :** _____

Date : _____ **Time :** _____

24 HOURS NURSE'S OVER SHEET

Date : _____

	Morning	Evening	Night
Introduction			
Situation			
Background			
Assessment			
Recommendation			

Name & ID of Nurse :	(Morning Duty)	Name & ID of Nurse :	(Evening Duty)	Name & ID of Nurse :	(Night Duty)
	(Evening Duty)		(Night Duty)		(Morning Duty)

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AH/MRD/FORM - 72

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AH/MRD/FORM - 71 Page no : 1

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NURSE CARE PLAN & RECORD

Date : _____ Time : _____ Shift : _____

GOALS	Care Plan	Time	Care Given	Sign
☐ Maintain airway patency and oxygenation ☐ Maintain fluid balance ☐ Relieve pain and discomfort ☐ Maintain good nutritional status ☐ Prevent infection ☐ Maintain skin integrity ☐ Ensure safety ☐ Improve activity tolerance ☐ Maintain personal hygiene ☐ Meet elimination needs ☐ Reduce anxiety ☐ Early ambulation ☐ Any Other _____				

Monitor Audible: ☐ Yes ☐ No ☐ NA
Airways : ☐ Clear ☐ Suction ☐ Nebulize/Steam
Hygiene : ☐ Mouth ☐ Eye ☐ Skin ☐ Perineum
Other Notes (If any) :

Time		Signature

Nurse Name : _____ Date : _____
Signature : _____ Time : _____

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AH/MRD/FORM - 5

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PLAN OF CARE

MLC: ☐ Yes ☐ No

Diagnosis with complaints in brief: _____

Conservative Treatment Plan (Mention Drugs) : _____

Operative (Name of surgery): Emergency Elective

Investigation required to include: _____

Cross Reference if required: _____

Expected Prognosis: _____

Possible Complications: _____

Other Restrictions / Instruction (If any): _____

Name of doctor: _____ Sign : _____

Name of Treating Consultant : _____ Sign: _____

Date: _____ Time: _____

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NUTRITIONAL ASSESSMENT FORM

Name of the Patient _____

Height _____ Weight _____ BMI _____

(If patient is bed ridden then take approximate height and weight according to patient or relative)

Subjective Information

Are you having any of the following problem? ***(Tick on applicable)***

- ☐ Constipation
- ☐ Diarrhea
- ☐ Difficulty in Chewing
- ☐ Difficulty in swallowing
- ☐ Mouth Sores
- ☐ Nausea
- ☐ Vomiting
- ☐ Not Applicable

Dietary Limitations (dislikes, cultural / religions)

Allergies and Food Sensitivities _____

Objective Information

Are you currently taking any Prescribed Medications (for Hypertension, Thyroid, Diabetes, Cardiac Disease) ? _____

Investigation Report

Glucose _____Albumin _____BUN _____Creatinine_____

Sodium _____Potassium _____ Cholesterol_____

Hemoglobin_____Triglycerides_____

Other _____

Assessment

Diet Recommendations on the Basis of Assessment

_____Kcal/Day_____Gm Protein / Day

Any Other Specifications_____

Diet Specifications

	FD	BD	SD	DD	SFD	SRD	HPD	PRD	LIQ	RTF	FFD
Consultant Advice											
Dietician's Advice											

Remarks

Pure Jain	Without Chilies	Without Tomato

Name & Sign. of Dietician _____ Date _____ Time _____

NUTRITIONAL ASSESSMENT FORM

Date						
Type of Food / Diet						
Notes						
Timings of Meals						
Dietician Sign.						
Medical Officer Sign.						
Nursing Name & Sign.						

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AH/MRD/FORM - 60 Page no : 1

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MEDICATION RECONCILIATION FORM

PATIENT NAME : _____ UHID : _____

TRANSFER FROM : _____ TRANSFER TO : _____

DATE : _____ TIME : _____

Drug Allergy:

List only those medications currently being taken (Medication name)	(Units) Dosage	Frequency	Route	Medications order on admission		Home medications continued at discharge?	
				Yes	No	Yes	No

Ref. of Doctor : _____ Sign : _____

DATE : _____ TIME : _____

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AH/MRD/FORM - 70

NURSING ADMISSION ASSESSMENT

(To be filled by nursing staff)

INITIAL ASSESMENT – To be filled by nursing staff within 15 minutes of admission

Date _____ Unit: _____
Patient Arrival Time: _____
Patient Assessment: _____
Height: _____ cm, Weight: _____ Kg
Unable to take Height/Weight due to _____
_____

Vital signs:
Temperature: ____ F; Pulse ____/min; RR ____/min
Blood Pressure _____ mm Hg SPO_2 _____

Denture: ☐ Yes ☐ No
☐ Upper
☐ Lower
☐ Par
☐ None

Eye aid:
☐ Yes ☐ No
☐ Glasses
☐ Contacts
☐ Other _____

Chief Complaint:_____
_____

Diagnosis: _____
_____

Identification Marks: _____
_____

Date: _____ **Time:** _____

History Informant ☐ Patient ☐ Other
Relationship: _____

Patient Arrival ☐ Ambulatory ☐ Wheelchair ☐ Stretcher ☐ Other

Patient Arrived from ☐ Home ☐ ER ☐ OPD ☐ Other

ID Band on: ☐ Yes ☐ No

Call Bell in reach: ☐ Yes ☐ No

Allergies: ☐ Yes ☐ No
☐ Drug
☐ Food
☐ Other

Reaction Previous Transfusion:
Yes ☐ No ☐
If Yes, describe _____
_____

Level of Consciousness (LOC)
☐ Alert
☐ Lethargic
☐ Responsive to Pain
☐ Unresponsive to Pain

Assistive Device
☐ Walker
☐ Cane
☐ Crutched
☐ Other

Name of Nursing Staff :_____ **Sign of Nursing Staff :** _____

COMPLETE ADMISSION ASSESSMENT – To be filled by nursing staff within 1 Hr of admission

Patient History

Hypertension	Yes/No	Asthma	Yes/No
Heart Disease	Yes/No	Stroke	Yes/No
Diabetes	Yes/No	Cancer	Yes/No
Dyslipidemia	Yes/No	Other Chronic Disease	Yes/No

Major Surgery

Family History: (Specify: F-Father, M-Mother, B-Brother, S-Sister)
(Please circle whichever is applicable e.g.
If father has H/O 10 yrs then, Yes/No F/10 yrs)

Hypertension	Yes/No	Asthma	Yes/No
Heart Disease	Yes/No	Stroke	Yes/No
Diabetes	Yes/No	Cancer	Yes/No
Dyslipidemia	Yes/No	Other Chronic Disease	Yes/No

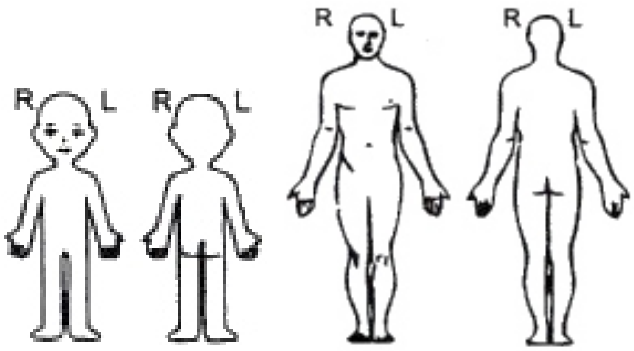
Major Surgery

Pediatric History

Diet Formula:
Bottle Feed: ☐ Yes ☐ No BF: ☐ Yes ☐ No
Diapers: ☐ Yes ☐ No
Toilet Training ☐ Yes ☐ No
Immunization Status: As per schedule ☐ Yes ☐ No
For Children under 2 years:
Head Circum_____ Chest Circum _____

Child Growth

Social ☐ Yes ☐ No Intellectual ☐ Yes ☐ No
Language ☐ Yes ☐ No Emo ☐ Yes ☐ No
Physical ☐ Yes ☐ No Behavior ☐ Yes ☐ No
Any Specific Remarks



History of Medication Patient Currently Taking

Name of Medication	Advice for continue	Advice By

Review of System

Respiratory
☐ Cough ☐ Emphysema
☐ SOB ☐ Asthma
☐ COPD ☐ Dyspnea
☐ NAD
☐ Other_____

Skin
☐ Intact ☐ Pale
☐ Dusky ☐ Dry
☐ Diaphoretic ☐ Mettled
☐ Cyanotic ☐ Flushed
☐ Warm ☐ Moist
☐ Other _____

Central Nervous System
☐ Dizziness ☐ Sedation
☐ Aphasia ☐ Ataxia
☐ Headache ☐ Slurred
☐ Speech ☐ NAD
☐ Paralysis ☐ Other

Genitourinary
☐ Pregnant ☐ Hematuria
☐ Amenorrhea ☐ Dysuria
☐ Dysuria ☐ Retention
☐ Veneral Disease ☐ Urinary Incontinence
☐ Enuresis ☐ Dysmenorrhea
☐ Having Menses
☐ Other ☐ NAD

Gastrointestinal
☐ Vomiting ☐ Blood in stool
☐ Nausea ☐ Diarrhea
☐ Ulcer ☐ Hepatitis
☐ NAD ☐ Other
☐ Constipation

Cardiovascular
☐ Angina ☐ Chest pain
☐ Peripheral ☐ Edema
☐ Hx ☐ HTN
☐ MI ☐ NAD
☐ Pacemaker ☐ SOB
☐ Heart Murmur ☐ Other

Musculoskeletal
☐ Fracture
☐ Joint Swelling / Pain
☐ Joint Stiffness
☐ Back pain
☐ Muscle Spasm
☐ Amputation
☐ NAD
☐ Others (Specify)

Others
☐ Hyperthyroid
☐ Diabetes
☐ HIV AIDS
☐ Hepatitis(Specify)
☐ Others (Specify)

Activities of Daily Living

	Usual Level	Level on admission
Feeding		
Bathing		
Toileting		
General Mobility / Gait		
Dressing / Grooming		

Level 0: Independent, requires no supervision, assistance or teaching
Level 1: Requires supervision and or teaching
Level 2: Requires at least minimum assistance from another person
Level 3: is dependent and does not participate

Vulnerable assessment

Sr. No.	Categories	Yes	No
1	Age more than 60 years		
2	Physically Challenged		
3	Mentally Challenged / ill		
4	Terminally ill		
5	Inability to speak		
6	Altered Consciousness		
7	Epileptic fit		
8	Medication related Consciousness defect		
9	Absence of relative attendant		
10	Immunocompromised / low immunity		
11	Age less than 12 years		

Vulnerability level (check which is applicable)
High Vulnerability (if more than 1 category is marked yes)
Low Vulnerability (if 1 category or factor is marked yes)

Nursing Plan of Care

General Nursing Care

- ☐ Sponging / Enema
- ☐ Mouth Care / Gargles
- ☐ Nail Care
- ☐ Back Care
- ☐ Vitals
- ☐ Drain
- ☐ Dressing
- ☐ Physiological Support

Curative

- ☐ IV Fluid
- ☐ Antibiotic
- ☐ Causative Treatment
- ☐ Continue Previous Medication
- ☐ Self Medication
- ☐ High Risk Medication

Safety Care (avoid fall risk)

- ☐ Side rail provision
- ☐ Low bed height
- ☐ Near to nursing station
- ☐ Continuous Monitoring
- ☐ Light & Sound Monitoring
- ☐ Language & Translator Required
- ☐ Full time attendant

Blood Transfusion ☐ Yes ☐ No

If yes, approx. requirement informed by doctor: _____

Any specific requirement: ☐ Yes ☐ No

If yes _____

Any procedure

_____

Name of Nursing Staff _____

Sign of Nursing Staff _____

Date: _____ Time: _____

Diabetic Care

- ☐ Oral Drug
- ☐ Insulin plan

Nutritional Care

- ☐ Ryle tube feeding
- ☐ Parental Nutrition
- ☐ Type of food _____

Physiotherapy Need ☐ Yes ☐ No

Pain Management ☐ Yes ☐ No

If Yes, Relieving Factor

- ☐ Rest
- ☐ Medication
- ☐ Other

Bedsore care

- ☐ Positioning changing
- ☐ Bedsore Dressing
- ☐ Education to patient / relative
- ☐ Application of cream

Infection Control Practices

- ☐ Regular Monitoring of site
- ☐ Hand washing

Any specific remarks:

Information provided by patient and family are correct and they are involved in patient care

Name of Patient/Relative:

_____

Sign of Patient/Relative:

_____

Relation with patient in case signature done by relative:

_____

BRADEN SCALE

Sensory Percep		Moisture		Degree of Ac		Mobility		Nutrition		Shear & Friction	
No Impairment	4	Rarely Moist	4	Walks Frequently	4	No Impairment	4	Excellent	4	No Problem	3
Slightly Limited	3	Occasionally Moist	3	Walks Occasionally	3	Slightly Limited	3	Adequate	3	Potential Problem	2
Very Limited	2	Very Moist	2	Chair Fast	2	Very Limited	2	In - Adequate	2	Problem Present	1
Completely Limited	1	Completely Moist	1	Bed Fast	1	Completely Limited	1	Very Poor	1		

Note : Severe Risk: Total score <=9, High Risk: Total Score 10-12, Moderate Risk: Total score: 13-14, Mild Risk: Total score 15-18, No risk: Total score 19-23 **Pressure sore:** Yes/No. If yes, area & level of pressure Sore: _____

Pain Assessment ☐ Yes ☐ No

Pain Location:

_____

Quality: Constant Intermittent
Character: Lacerating Burning
Radiating
After Daily Routine: ☐ Yes ☐ No
Sleep: ☐ Yes ☐ No

Fall Risk Assessment:

Parameter	Parameter Details	Scale
History of Fall	History of previous fall	2
Physical Status	Fa	2
	Dizziness / Balance Problems	1
	Impaired Mobility	1
	Sensory Impairment	1
	Seizure Disorder	1
	Altera	1
Mental Status	Confused (illogical thinking)	2
	Impaired Memory / Judgement	2
	Disoriented to person / place / time	2
	Lack of familiarity with immediate surroundings	1
	Inability to understand / follow instructions	1

Parameter	Parameter Details	Scale
Medication	Drugs that have diuretic effect	1
	Drugs that alter throughout process and / or create hypertensive effect (Narcotic, Sedatives, Psychotropic, Hypnotic, Tranquilizers, Anti- Hypertensive)	1
	Drugs that increase GI mo (Laxatives, Enemas, Chathartics)	1
	Multiple drugs from different drugs classifications	1

Note: Potential risk for falls : Total score = > 5

Total score _____

Note: Hi Risk = 16-21 Medium Risk = 12-15
Low Risk = 5-11

GCS Score (Circle the score whichever is applicable)

Category	Instruction	Score
EYE OPENING	No Response	1
	To Pain	2
	To Speech	3
	Spontaneous	4
BEST MOTOR RESPONSE	No Response	1
	Extension	2
	Flexion – Abnormal	3
	Flexion – Withdrawal	4
	Localizes Pain	5
	Obeys a Siple Response	6

Category	Instruction	Score
BEST VERBAL RESPONSE	No Verbal Response	1
	Response within Comprehensible Sound	2
	Inappropriate words in comprehensible	3
	Confused	4
	Oriented	5

Note: Best Response = 15 (Mild 13-15)
Comatose Client = 8 or Less (Moderate 8-12)
Totally Unresponsive = 3 (Severe 3-7)

Total score _____

NUTRITIONAL SCREENING CIRCLE THE APPROPRIATE RESPONSE THEN TOTAL THE SCORES

1. Does the patient had any unintentional weight loss / weight gain (adults only) Tick Here.
>2 Kgs ☐ If Yes (1) > 4 Kgs ☐ If Yes (2) > 8 Kgs ☐ If Yes (3)
2. Does the patient appear emaciated or BMI is below 18 ? ☐ If Yes (2)
3. Does patient look obese or BMI is above 35 ? ☐ If Yes (2)
4. Does patient had any GI symptoms like nausea, diarrhoea, anorexia, Vomiting, Chewing/swallowing difficulty in last 2 weeks ? ☐ If Yes (2)
5. Does the patient have any of the following ?
External tube feeding ☐ If Yes (3) Parenteral tube feeding ☐ If Yes (3) Multiple food allergies ☐ If Yes (3)
6. Is the patient demented, unresponsive or inappropriate in responding ? ☐ If Yes (2)
Total Score _____

(For total scores equal to or greater than 3 further nutritional assessment needs to be carried out by the dietician).

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CANCER ASSOCIATES
CONSULTANT CANCER SURGEONS

AH/MRD/FORM - 10

ADDRESSOGRAPH
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PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

ADMISSION, HISTORY AND PHYSICAL ASSESSMENT FORM

(To be filled up by Medical Officer/Registrar)

Medicolegal Case : ☐ Yes ☐ No Date :.....Time.....

For MLC Patient (only)

1. Police Station Name :..... Two Identification Marks
2. Constable Name :..... (1) .....
3. Buckle No :..... (2) .....
4. Informd by. :..... 5. Date : .....

Name of the Patient : ..... Age :..... Sex : ☐M ☐F Height : .....

Name of MO/ Registrar Taking History : ..... Weight : .....

Allergies : ☐ None ☐ Yes (If Yes, describe)

Chief Complaints with Origin, Duration and Progress

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

Investigation :

.....

.....

.....

.....

.....

Past History :

Hypertension Since : .....

On Medication 1).....
2).....
3).....

Under Treatment of Dr.....

Ischaemic Heart Disease Since : .....

On Medication 1).....
2).....
3).....

Under Treatment of Dr.....

Any Intervention done.....

Diabetes Since : .....

On Medication 1).....
2).....
3).....

Under Treatment of Dr.....

Tuberculosis When : .....

Any Other P/H.....
.....

Any Other Medication.....
.....
.....

P/H of Operation / Hospitalization

Diagnosis / Operation .....

Year .....

Duration of Hospitalization .....

Others .....

Personal History

Diet		Smoking	Yes/No	since...../.....per day
Appetite		Alcohol	Yes/No	since...../.....(freq.)
Sleep		Drugs	Yes/No	since...../.....(freq.)
Micturition		Tobacco	Yes/No	since...../.....(freq.)
Bowel Habits		Any other habit		

Family History: (Specify: F-Father, M-Mother, B-Brother, S-Sister)
(PI. Circle whichever is applicable e.g. If Father has h/o 10 yrs then, (yes) / No F/10 yrs)

Hypertension	Yes/No	Asthima	Yes/No
Heart Habits	Yes/No	Stroke	Yes/No
Diabetes	Yes/No	Arthiritis/Gout	Yes/No
Tubrculosis	Yes/No	Cancer	Yes/No
Epilepsy	Yes/No	Other Chronic disease	Yes/No

FOR FEMALES :

Menstrual History : L.M.P. Date :_____ Regular/Irregular:_____

Obestetric History : L.D._____

Abortion:_____

Others : _____

Declaration by the Informant (Patient/Relative/ Accompanying person I hereby declare that the facts recorded above are based on my narration and are accurate to the best of my knowledge and I would not request anyone to make changes in the details provided by me.	Name of Informant (Patient/ Relative/ Accompanying Person)..... Relationship with patient:..... Signature..... Date..... Time.....
માહિતી આપનારનું નિવેદન (દર્દી દર્દીના સગા સાથે આવનાર (વ્યક્તિ) હું સ્વીકારૂં છું કે જે માહિતી અહી નોંધવામાં આવેલ છે, તે મેં આપેલ જાણકારી પ્રમાણે છે. તથા મારી જાણ પ્રમાણે એ એકદમ સાચી છે. તથા આપેલ માહિતી બદલાવવા માટે હું કોઈને પણ વિનંતી નહી કરૂં.	માહિતી આપનારનું નામ (દર્દી દર્દીના સગા સાથે આવનાર વ્યક્તિ)_____ દર્દી સાથેનું સગપણ_____સહી_____ તારીખ_____ સમય_____

जाणकारी देनेवाले का निवेदन (मरीज/मरीज के संबंधी / साथ आनेवाला व्यक्ति) में कबुल करता हुं की जो जानकारी यहाँ दर्ज करने में आई है वो मेरी जानकारी के हिसाब से बीलकुल सही है | और दी गई जानकारी बदल ने के लिए में कीसी को बीनती नहीं करुँगा |	जाणकारी देनेवाले का नाम (मरीज/मरीज के संबंधी / साथ आने वाले व्यक्ति) _____ मरीज के साथ संबंध _____हस्ताक्षर _____ दिनांक _____ समय _____

Physical Examination :

Vital Signs:

Tem :_____°F Pulse : _____/min R/R.: _____/min B.P. : _____mm Hg

Pain Score _____ (on Visual Analogue Scale) SPO2 : _____ %

General Appearance:

☐ Anemia ☐ Cyanosis ☐ Jaundice ☐ Generalized Lymphadenopathy ☐ Pedal oedema

General Examination: Head / Eyes / Ears / Nose / Thrpat / Neck

Systemic Examination

Respiratory System

Air-entry : equal/ unequal (with details)

Cardiovascular System

Murmur : No Murmur / Systolic / Diastolic

S_1 S_2:

Arrhythmia :

Musculoskeletal System

Extremities/Spine

Abdomen

Tenderness : Yes / No

Distension : Yes / No

Palpation :

Peristalsis :

Centrral Nervous System

Conscious / unconscious

Power :

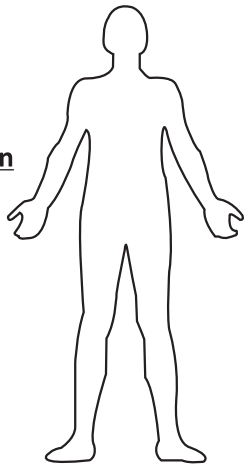
Reflexes :

GCS :

Local Examination :

Skin

For Injury / Burns description



Provisional Diagnosis :_____

_____

_____

_____ _____

Name of Medical Officer/Registrar Sign of Meducal Officer

_____ _____

Date Time

Medical Officer / Registrar's notes : S/B .....Date.....Time.....

_____

_____

_____

_____

_____

_____

_____

Signature : _____

Plan of care during Hospitalization :

_____

_____

_____

_____

_____

_____

_____

Signature : _____

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CANCER ASSOCIATES
CONSULTANT CANCER SURGEONS

ADDRESSOGRAPH
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PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

AH/MRD/FORM - 86

DOCTOR'S HANDOVER & TAKEOVER FORM

Ward/Unit :

	Date :	Referrals Special Investigations Special Instructions		**Date :**	Referrals Special Investigations Special Instructions
	Important Information Problems Interventions Pending Action	Taking Over Doctor Signature _____ Full Name _____		**Important Information** Problems Interventions Pending Action	Taking Over Doctor Signature _____ Full Name _____
	Handing Over Doctor Signature _____ Full Name _____			Handing Over Doctor Signature _____ Full Name _____	

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Ward/Unit :

	Date :	Referrals Special Investigations Special Instructions		Taking Over Doctor Signature _____ Full Name _____	
	Important Information Problems Interventions Pending Action			Handing Over Doctor Signature _____ Full Name _____	
	Date :	Referrals Special Investigations Special Instructions		Taking Over Doctor Signature _____ Full Name _____	
	Important Information Problems Interventions Pending Action			Handing Over Doctor Signature _____ Full Name _____	

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AH/MRD/FORM - 86

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Ward/Unit :

	Date :	Referrals Special Investigations Special Instructions		Taking Over Doctor Signature _____ Full Name _____	
	Important Information Problems Interventions Pending Action			Handing Over Doctor Signature _____ Full Name _____	
	Date :	Referrals Special Investigations Special Instructions		Taking Over Doctor Signature _____ Full Name _____	
	Important Information Problems Interventions Pending Action			Handing Over Doctor Signature _____ Full Name _____	

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AH/MRD/FORM - 24

INVESTIGATIONS

Blood Group:_____ HIV I & II_____ HbsAg_____ Sheet No.: _____

Date								
Hb								
TC								
DC								
ESR								
Platelet Count								
Peripheral Smear								
BT								
CT								
PT / INR								
APTT								
Stool For OB								
FBS								
PPBS								
RBS								
S. Bilirubin (T)								
Direct - Bilirubin								
Indirect - Bilrubin								
SGOT								
SGPT								
GGT								
ALP								
S. Protein (T)								
S. Albumin								
S. Globulin								
AG Ratio								
S. Amylase								
S. Lipase								
S. Urea								
S. Creat								
S. Na								
S. K								
S. Cl								
S. Ca								
S. Mg								
S. Phosphate								
S. Ammonia								
S. Uric Acid								
TSH								
CPK Total								
CPK MB								
Trop-T								
Trop-I								
Widal								
LDH								
Fibrinogen								
D Dimer								
CRP								
ProcalcitoniN								
ABG-PH								
PCO_2								
PO_2								
TCO_2								
HCO_3								
Base Excess								
SaO_2								
AnionGap								

RADIOLOGICAL INVESTIGATION

Date				
ECG				
X-RAY				
USG				
CT Scan				
MRI				
ECHO				
Carotid/peripheral doppler				
Other				

MICROBIOLOGICAL INVESTIGATIONS

	Routine / Micro	Gram Stain	Culture Sensitivity
Sputum / ET			
Urine			
Blood			
CSF			
Pleural Fluid			
Ascitic Fluid			
Other			

OTHER INVESTIGATIONS

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CANCER ASSOCIATES
CONSULTANT CANCER SURGEONS

AH/MRD/FORM - 47 Page no : 1

ADDRESSOGRAPH
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TRANSFER FORM TO INTERNAL DEPARTMENTS

Patient Name _____ Date: _____ Time: _____ UHID No._____

Transferring Unit Nurse _____ Employee ID No. _____

Receiving Unit Nurse _____ Employee ID No._____

Doctor's Name _____ Diagnosis _____ Post Operation Days _____

Transfer From	Transfer To	Time of Transfer

Reason for Transfer (Filled by Doctor)

Current Vitals

Pulse _____ BP _____ RP _____

Temperature _____ HR _____ Pain (0-10) _____ SPO_2 _____

At Risk For

Falls : ☐ Yes ☐ No **Seizures :** ☐ Yes ☐ No **Vulnerable :** ☐ Yes ☐ No **Aspirations :** ☐ Yes ☐ No

Q_2 Requirement : ☐ Yes ☐ No **If Yes :** Mask / Nasal Prong **Vasopressors :**

Brief Summary of the Patient

Admission History _____

Diagnosis / Provisional Diagnosis _____

Treatment Given _____

Transfer Instructions

_____

Admit in

ICU ☐ CCU ☐ General Ward ☐ Private Room ☐ SICU ☐ Other ☐

CLINICAL HANDOVER _____

Doctor Name, Signature, Date & Time of (Accompanying The Pt.)

Doctor Name, Signature, Date & Time of (Receiving The Pt.)

Case Sheet Yes _____ No _____ **All Investigations** Yes _____ No _____

Medication if Any Yes _____ No _____ **Personal Belonging** Yes _____ No _____

Primary Treating Consultant / Doctor's Name _____

*(Inform consultant after admission in respective (WARD/UNIT)

Sr. No.	Documents / Consent	Transferring Unit Nurse	Receiving Unit Nurse	Remarks
1.	Admission Dept's Permission Taken	☐	☐	
2.	Receiving bed prepared & ward Nurse informed	☐	☐	
3.	Handed over case file complete with history & progress notes - updated all charts	☐	☐	
4.	Handed over all Pathology reports	☐	☐	
5.	Handed over X-Ray plates with report	☐ No of Plates.....	☐ No of Plates.....	
6.	Handed over CT Scan / MRI Plates with Report	☐	☐	
7.	Patient's own case file	☐	☐	
8.	Any CD (Angiography etc.)	☐	☐	
9.	Patient's Valuables Including Glasses and Dentures	☐	☐	
10.	Handed over No of Medicines - to count & list on the back	☐	☐	
11.	Handed over No of Medical items like Eau-d-colonge, Powder etc.	☐	☐	
12.	Relative Informed Name of Relative : _____ Signature : _____	☐	☐	
13.	Patient Dress	☐	☐	
14.	CVP / DLC / Drain / Foley's	☐	☐	
15.	Bed Sore	☐	☐	
16.	Diet	☐	☐	
17.	Other	☐	☐	

Allergies (To be Filled by Nursing Staff) _____

Drug Yes _____ No _____ **Name of The Drug** _____

Sp. Note _____

Transferring Nurse _____ **Receiving Nurse** _____

Transferring Nurse Name _____ **Receiving Nurse Name** _____

Transferring Nurse Sign _____ **Receiving Nurse Sign** _____

Date _____ **Date** _____

Time _____ **Time** _____

TRANSFER FORM TO INTERNAL DEPARTMENTS

MEDICINE LIST

Sr. No.	Medicine List	Strength	Dose / Frequency	Days

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CANCER ASSOCIATES
CONSULTANT CANCER SURGEONS

AH/MRD/FORM - 57 Page no : 1

ADDRESSOGRAPH
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PROGRESS NOTE

Date / Time	Notes	Name & Sign. M.O.

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CANCER ASSOCIATES
CONSULTANT CANCER SURGEONS

AH/MRD/FORM - 28 Page no : 1

ADDRESSOGRAPH
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CONSULTANT'S NOTES

Date	Time	Notes	Sign. of Consultant

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CANCER ASSOCIATES
CONSULTANT CANCER SURGEONS

ADDRESSOGRAPH
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AH/MRD/FORM - 49 Page no : 1

HIV TEST INFORMATION AND CONSENT

Name : _____ sex: _____ Age: _____

UHID No.: _____ OPD / INDOOR No : _____ Ward: _____ Bed: _____

Referred by DR.: _____

Signature & Name (Consultant/Registrar/MO/PCC) : _____

Indication for HIV Testing : _____

My doctor informed me that the Human Immunodeficiency Virus (HIV) antigen test will be done on a sample of my blood. I understand that this test shows the presence or absence of the HIV (AIDS) virus. I understand that a negative test does not necessarily mean that I have not been infected with HIV. I have had a chance to ask questions about the meaning of these tests. I have been informed that I will be offered counseling from trained individuals if the results of the test show that I have, or that I might have, the HIV virus . I will also be told if my test is negative (shows nothing). If the test is unsure, another test may be suggested in the next six to twelve months.

Confidentiality :-

I understand that my test result will be kept confidential to the full extent required by law. I understand that particular care is being taken to maintain my records in a secure manner. There are five exceptions to confidentiality. The exceptions allow release to the local health authority as required, the ordering Doctor, health care personnel having a legitimate need to know, the subject or the person giving legal consent, and the spouse if the patient tests positive.

I understand that I can refuse consent and that testing will only be done without my consent for management of an accidental exposure of a health care worker to my blood or body fluids.

☐ I do consent to the test for the HIV antibody / antigen.

☐ I do not consent to the test for the HIV antibody / antigen

Patient is unable to sign the consent as certified by the Doctor.

A witness should sign. below if the patient is unable to sign. but has indicated his or her consent. Young people, / children may also like a parent to sign. here.

Date : _____

Name of the Patient: _____ Sign. of the Patient : _____

Name of the Relative: _____ Sign. of the Relative _____

Relation with Patient: _____ Date/Time: _____

ATTENDING CONSULTANTS STATEMENT

As this patient's Attending Consultant, I certify under penalty of law that I have obtained the consent of this patient for the HIV antibody/antigen test, or the performance of the test is within the legal criteria for testing without the patient's consent as stated above and in the policy and procedure. I also certify that I will inform the patient of any test results and arrange for post-test counselling if the test is positive or indeterminate. I have recorded the indications for this test and my plan for counselling in the patient's medical record.

Attending Consultant's Name : _____ Attending Consultant's Signature : _____

Date/Time: _____

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CANCER ASSOCIATES
CONSULTANT CANCER SURGEONS

AH/MRD/FORM - 49 Page no : 1

ADDRESSOGRAPH
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PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

HIV CONSENT

નામ _____ જાતી _____ ઉંમર _____

UHID No.: _____ OPD / INDOOR No : _____ વોર્ડ _____ બેડ _____

રેફ. ડોકટર _____

સહી. (કન્સલટન્ટ/રજીસ્ટ્રાર/MO/PCO) _____

ટેસ્ટ કરવા માટેનું કારણ : _____

એચ. આઈ.વી. ટેસ્ટ માટે માહિતી અને સંમતી

દર્દીની માહિતી અને સંમતિ :

મારા ડૉ. દ્વારા મને જણાવવામાં આવ્યું છે કે એઈડ્ઝનાં એચ.આઈ.વી. વાયરસ માટે મારા લોહીના નમુનાની તપાસ કરવામાં આવશે. આ તપાસનું પરિણામ HIV (AIDS) ની હયાતી અથવા HIV વિષાણુ ઓની ગેરહાજરી આવી શકે.જો મારા લોહીમાં આ તપાસનાં અંતે કંઈ પણ HIV સબંધી ન મળે તેનો અર્થ એવો નથી કે હું HIV વિષાણુઓથી મુક્ત હોઈ શકુ.આ તપાસ સબંધી તમામ પ્રશ્નો પૂછવાનો મને સંપૂર્ણ અધિકાર અને તક આપવામાં આવેલ છે.જો મારા લોહીમાં HIV ના વિશાણુઓ હાજર અથવા HIV વિશાણુ હોવાની શક્યતા હશે, તો નિષ્ણાંત તબીબો અને તાલીમ પામેલા માર્ગદર્શકો મને મદદરૂપ થશે.એવું પણ મને જણાવવામાં આવ્યું છે.જો મારા HIV ટેસ્ટનું પરિણામ નેગેટીવ આવશે તો તે પણ મને જાણ કરવામાં આવશે.જો મારા HIV ટેસ્ટનું પરિણામ અનિશ્ચિત હશે,તો 11 કે ૧૨ મહિના બાદ મને ફરીથી એકવાર તપાસ કરવાનું જણાવવામાં આવ્યું છે. હું સારી રીતે સમજુ છું કે HIV ટેસ્ટ અંગેની માહિતી કાયદાકીય જરૂરિયાત અનુસાર ગુપ્ત રાખવામાં આવશે.મારા તપાસના તમામ રેકોર્ડ પણ સલામત સ્થાને સુરક્ષીત રાખવામાં આવશે.આમાં નીચેના પાંચ અપવાદો પણ હોઈ શકે. જો તપાસ માં HIV વિશાણુઓની હાજરી હશે, તો નીચે મુજબની વ્યકિતઓને મારી HIV ટેસ્ટની માહિતી આપવામાં આવશે.

(1) તબીબી અધિકારી
(2) સારવાર આપનાર તબીબ
(3) દર્દીની સાથે સંભાળ રાખનાર વ્યક્તિઓ જે મને તપાસના પરિણામ જાણવાની જરૂરીયાત હશે.
(4) દર્દીની સારવાર માટેની સંમતી આપનાર વ્યક્તિ
(5) પતિ કે પત્ની

હું સારી રીતે જાણું છું કે હું આ તપાસનો ઈન્કાર કરી શકુ, પરતુ મારી સાથે મારી સારવારમાં સંકળાયેલ હોસ્પિટલના ડૉક્ટર અને અન્ય સ્ટાફ મારા લોહી અથવા Body Fluids ના સંપર્કમાં આવશે તો મારી HIV ની તપાસ મારી નામરજી હોય તો પણ કરવામાં આવશે.

☐ હું એચ. આઈ. વી. ટેસ્ટ માટે સંમતિ આપું છું.
☐ મારી એચ. આઈ. વી. ટેસ્ટ માટે સંમતિ નથી.

દર્દીનું નામ : _____ દર્દીની સહી : _____

સગાનું નામ : _____ સગાની નામ : _____

દર્દી સાથેનો સંબંધ : _____ તારીખ : _____

તબીબ નું નિવેદન

દર્દીની સારવાર આપનાર તબીબ તરીકે હું તમામ કાનુની જવાબદારી સાથે જાહેર કરું છું કે દર્દીની સંપૂર્ણ સંમતિ લઈને અથવા દર્દીના સબંધીત સગાઓને ઉપરોકત તમામ વિધિઓ અને તેના પરિણામો સમજાવ્યા બાદ દર્દી માટે કાનુની રૂપે લોહીની એઈડ્ઝ એચ. આઈ. વી. અંગે તપાસ કરાવી છે. આથી હું જાહેર કરું છું કે હું તપાસ પછીના તમામ પરિણામોથી દર્દીને અવશ્ય વાકેફ કરીશ. અને ડૉકટરે નોંધવાનાં રેકર્ડમાં આ સંબધીત તમામ વિગતોની મેં નોંધ લીધી છે.

ડૉકટર નું નામ _____ ડૉકટરની સહી _____

તારીખ/સમય _____

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CANCER ASSOCIATES
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ADDRESSOGRAPH
If ADDRESSOGRAPH not Available, Write
PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

AH/MRD/FORM - 15

એનેસ્થેસિયા સંમતિપત્રક

હું _____ સ્વીકારું છું કે મારા ડોકટરે (સર્જન અને એનેસ્થેસિયોલોજિસ્ટ) દ્વારા મને શસ્ત્રક્રિયા (સર્જરી), નિદાન અને સારવાર બાબતમાં સંપૂર્ણપણે સમજાવવામાં આવ્યું છે. મારા ડોકટરે મને સારવારનો તમામ ખર્ચ, સારવારના જોખમો અને સારવારના અન્ય વિકલ્પ તથા સારવાર ન કરવામાં આવે તો તેના પરિણામો વિશે પણ વિગતવાર મારી સમજમાં આવે તે ભાષામાં સમજાવ્યું છે.

ડોકટરે દ્વારા મને એ પણ સમજાવવામાં આવ્યું છે કે દરેક પ્રકારના એનેસ્થોસિયા સાથે કઈકને કઈક જોખમો કે આડઅસર સંકળાયેલા છે અને સારવાર સાથે સંકળાયેલા કોઈ પણ પ્રકારના પરિણામો બાબતમાં વચન કે બાંહેધરી આપવી શકાય નથી. જોકે જવલ્લે જ થતા એનેસ્થોસિયા સાથે સંકળાયેલા ગંભીર એને અણધાર્યા જોખમો જેવે કે ચેપ લાગવો-ઈન્ફેકશન, રકતસ્ત્રાવ, દવાની આડઅસર, સંવેદના - સ્પર્શ ચેતના ગુમાવવી, લકવો, સ્ટ્રોક, મગજમાં નુકસાન, હાર્ટ એટેક અથવા મૃત્યૂ સંકળાયેલા છે. હું સમજું છું કે આ જોખમો દરેક પ્રકારના એનેસ્થોસિયા સાથે સંકળાયેલ છે અને આ ઉપરાંત ચોક્કસ પ્રકારમા એનેસ્થોસિયા સાથે સંકળાયેલા જોખમો નીચે મુજબ વર્ણવેલા છે.

હું સમજું છું કે પ્રસ્તુત શસ્ત્રક્રિયા કે શલ્યક્રિયા માટે મારી શારીરિક પરિસ્થિતિ તેમજ બીજા અન્ય પરિબળોને ધ્યાનમાં રાખીને નીચે પ્રમાણેના નામાંકિત એનેસ્થોસિયા ચોક્કસ પદ્ધતિથી આપવામાં આવશે. એનેસ્થોસિયાના વિકલ્પની પસંદગી તે, મારા ડોકટર દ્વારા મને થનારી શસ્ત્રક્રિયા કે શલ્યક્રિયા, મારી ઈચ્છા અને ડોકટરની અંગત પસંદગી પર આધારીત છે.

મને સમજાવવામાં આવ્યું છે કે લોકલ એનેસ્થોસિયા (ઘેનની અસર વગર કે ઘેનની અસર હેઠળ) ઘણી વાર કારગત નીવડતા નથી અને પદ્ધતિ બદલવી પડે છે. જેમાં જનરલ એનેસ્થોસિયા (સંપૂર્ણ બેભાન અવસ્થા)નો પણ સમાવેશ થાય છે.

☐ જનરલ એનેસ્થોસિયા	અપેક્ષિત પરિણામો	સંપૂર્ણ બેભાન અવસ્થા સુનિશ્ચિત કરવા માટે શ્વાસનળીમાં ટ્યુહનળીકે અન્ય હવા જઈ શકે તેવું સાધન મુકવું
	પદ્ધતિ	દવાને સીધી મુખ્ય રકત પ્રવાહમાં આપવામાં આવે,સૂઘાડવામાં આવે કે અન્ય કોઈ રીતે આપવામાં આવે જેમ કે સ્નાયુમાં
	જોખમો	મોઢુ કે ગળામાં દુ:ખાવો, અવાજ ઘોઘરો થવો, ખારાશ, મોં અને દાંત પર ઈજા, બેભાન અવસ્થામાં સંવેદના આવવી, રકતવાહીનીને ઈજા, એસ્પીરેશન ન્યુમોનિયા
☐ સ્પાઈનલ અથવા એપીડ્યુરલ ☐ ઘેનની અસર હેઠળ ☐ ઘેનની અસર વગર	અપેક્ષિત પરિણામો	શરીરના નીચેના ભાગમાં હલન ચલન, સંવેદના ગુમાવવી અથવા તેમાં ઘટાડો થવો.
	પદ્ધતિ	કમરમાં સોય દ્વારા દવાનો કરોડરજ્જુ કે તેના અંદરના ભાગમાં કે તેની આસપાસના વિસ્તારમાં આપવી(એપીડ્યુરલ સ્પેસ)
	જોખમો	એનેસ્થેસિયા માટે ઉપયોગમાં લેવાયેલી દવાની એલર્જી, કરોડસ્તંભની આસપાસ લોહી ભરાઈ જવું, કાનમાં ભણકારા, અશકિત, અનિયમિત ધબકારા, બ્લડ પ્રેશરમાં ઘટાડો થવો, કરોડરજ્જુ કે મગજમાં ઇન્ફેકશન, ચેતાઓમાં ઈજા, આચકી કે તાણ, માથામાં સખત દુ:ખાવો વગેરે
☐ મુખ્ય અથવા નજીવી નસો બ્લોક થવી ☐ ઘેનની અસર હેઠળ ☐ ઘેનની અસર વગર	અપેક્ષિત પરિણામો	ચોકકસ અંગ કે અન્ય ભાગમાં થોડા સમય માટે હલનચલન કે સંવેદના ગુમાવવી.
	પદ્ધતિ	સર્જરી જયાં કરવાની હોય તે ભાગ પાસે સંવેદના આપતી નસ પાસે દવા આપવી
	જોખમો	ચેપ, સોજો, સતત ઝણઝણાટી, વાઈ, ઈન્જેકશનની જગ્યા પર દુ:ખાવો રહી જવો, કરતવાહિનીને ઈજા, ફેફસાને ઈજા, અચાનક નિશ્ચેતન થઈ જવું
☐ ઈન્ટ્રાવીનસ રીજીયોનલ એનેસ્થેસિયા ☐ ઘેનની અસર હેઠળ ☐ ઘેનની અસર વગર	અપેક્ષિત પરિણામો	ચોકકસ અંગ કે અન્ય ભાગમાં થોડા સમય માટે હલનચલન કે સંવેદના ગુમાવવી.
	પદ્ધતિ	ટૉર્નીકવેટના ઉપયોગથી હાથ કે પગની રકતવાહિનીમાં દવા આપવી
	જોખમો	ચેપ, સોજો, નબળાઈ, એલર્જીનું રિએકશન, સતત ઝણઝણાટી, વાઈ, ઈન્જેકશનની જગ્યા પર દુ ખાવો રહી જવો, રકતવાહિનીને ઈજા, અચાનક નિશ્ચેતન થઈ જવું, અનિયમિત ધબકારા, એનાફાઈલેક્સીસ.

☐ મોનીટર્ડ એમેસ્થેસિયા કેર (ઘેનની અસર હેઠળ)	અપેક્ષિત પરિણામો	ઉત્તેજના અને દુ:ખાવામાં ઘટાડો, આંશિક અથવા સંપૂર્ણ સ્મૃતિભ્રંશ
	પદ્ધતિ	અર્ધબેભાન અવસ્થા લાવવા માટે દવાને રકતવાહિનીમાં આપવી, દવા સુઘાડવી અથવા બીજી કોઈ રીતે દવા આપવી
	જોખમો	બેભાન અવસ્થા, શ્વાસ રૂંધાવો, રકતવાહિનીને ઈજા
☐ મોનીટર્ડ એમેસ્થેસિયા કેર (ઘેનની અસર વગર)	અપેક્ષિત પરિણામો	વાઈટલ પેરામિટરનું નિરીક્ષણ અને જરૂર પડે તો એનેસ્થેસિયા ઈન્ટરવેન્શન
	પદ્ધતિ	દર્દીની જરૂરિયાત મુજબ
	જોખમો	ઉત્તેજના, સંવેદના વધવી, બેચેની થવી વગેરે

હું અહીં, ઉપર દર્શાવેલી એનેસ્થેસિયા સેવા માટે સંપૂર્ણ સંમતિ આપું છું એને એનેસ્શેસિયા ટીમના એક અથવા તેથી વધુ સભ્યોને (ડો. _____) એનેસ્શેસિયા આપવા માટે સંમતિ આપું છું. આ તમામ ડોકટરો ડોસ્પિટલમાં એનેસ્શેસિયા સર્વિસ પૂરી પાડે છે. જો તેમને યોગ્ય લાગશે તો જરૂરિયાત પ્રમાણે, એનેસ્શેસિયાના અન્ય વિકલ્પોનો પણ ઉપયોગ કરવા માટે હું સંમતિ આપું છું.

મને અમને સર્જરી પછીના દુખાવાને અંકુશમાં રાખવા વિશેની પદ્ધતિની સમજણ આપવામાં આવી છે.

મને અમને ચોક્કસ પ્રકારના સંલગ્ન રોગોના કારણે સંકળાયેલા વધારાના કે ચોકકસ જોખમો વિશે મને-અમને-અમારા દર્દીને સમજ આપવામાં આવી છે.

મારા અમારા દર્દીમાં ઓળખાયેલ સંલગ્ન રોગ, જેના લીધે સારવારનું જોખમ વધે છે તે નીચે મુજબ છે.....

1. _____
2. _____
3. _____
4. _____
5. _____

હું મારી સહી (દર્દી અથવા જવાબદાર પ્રતિનિધિ) સાથે સંમતિ આપું છું કે:

1. હું સ્વીકારું છું ક્ મને ઉપરોકત સારવાર પ્રક્રિયાને સમજવાની અને મારા ડોકટર અથવા તેમના દ્વારા નિયુકત વ્યકિત સાથે મને સમજાય તે ભાષામાં ચર્ચા વિચારણા કરવાની તક મળી છે.
2. મારા દરેક સવાલના જવાબ સંતોષકારક રીતે આપવામાં આવ્યા છે અને આપેલા સંમતિપત્રમાં આપેલી તમામ માહિતીથી હું વાકેફ છું.
3. હું પ્રસ્તાવિત સારવાર, તેના જોખમો, ફાયદા, તકલીફો, સંભવિત વિકલ્પ, શસ્ત્રક્રિયા કાર્યવિધિ સંબંધિત સફળતા અને રિકવરી બાબતની સમસ્યાઓ, અને સારવાર ન કરવાના શક્ય પરિણામો સમજ્યા પછી કોઈના દબાણ કે બળજબરી વિના સંપુર્ણ સ્વસ્થતાપુર્વક સંમતિ આપું છું.

દર્દીનું / દર્દીના સગાનું નામ	દર્દી / દર્દી સાથેનો સંબંધ	દર્દીની / દર્દીના સગાની સહી અથવા અંગુઠાનું નિશાન
કન્સલ્ટન્ટનું નામ		કન્સલ્ટન્ટની સહી
સાક્ષી (સ્ટાફ નર્સ) નું નામ		સાક્ષી (સ્ટાફ નર્સ) ની સહી
તારીખ : _____		સમય : _____

મધ્યસ્થીનું નિવેદન

મે દર્દીના સંમતિપત્રનું _____ (દર્દીની ભાષા અહીં લખો) ભાષાંતર કર્યું છે. અને તબીબ દ્વારા દર્દી અથવા દર્દીના સબંધિઓને કે જે દર્દી વતી નિર્ણય લઈ શકે છે તેમને આપેલી કોઈ પણ મૌખિક અને લેખિત માહિતીની જોગવાઈમાં મદદ કરી છે.

મધ્યસ્થીનું નામ : _____ સહી : _____

તારીખ : _____ સમય : _____

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CONSENT FOR ANAESTHESIA

I _____, acknowledge that my doctors, (surgeon & anaesthesiologist) have explained to me in detail about my operation, diagnostic or treatment procedure. My doctor has explained the cost, risks of the procedure, advised me of alternative treatments and the risk of not having procedure and told me about the expected outcome and what could happed if my condition remains untreated in a language that I understand.

It has been explained to me that all forms of anaesthesia involve some risk and no guarantees or promises can be made concerning the results of my procedure or treatment. Although rare, Unexpected severe complications with anaesthesia can occur and include the remote possibility of infection, blessing, drug reactions, blood clots, loss of limb function, paralysis, stroke, brain damage, heart attack or health. I understand that these risk apply to all forms of anaesthesia and that additional or specific risks have been identified below as the may apply to a specific type of anaesthesia. I understand that the type(s) of anaesthesia service checked below will be used for my procedure and and that the anaesthetic technique to be used is determined by many factors including my physical condition, the type of procedure, doctor's preference as well as my own desire. It has been explained to me that sometimes an anaesthesia technique which involves the use of general / local / regional anaesthesia, with or without sedation, may not succeed completely and therefor another technique may have to be used.

☐ General Anaesthesia	Expected Result	Total unconscious state, Possible placement of a tube into the windpipe or other device to secure airway.
	Technique	Drug injected into the bloodstrem, breathed into the lungs, or by other route, Eg: IM
	Risks	Possinility of mouth or throat pain, hoarseness, injury to mouth or teeth, awareness under anaesthesia, injury to blood vessels, aspiration pneumonia, vomiting.
☐ Spinal or Epidural analgesia / anaesthesia ☐ With Sedation ☐ Without Sedation	Expected Result	Temporary decrease or loss of felling and / or movement to lower part of the body (Unilateral / Bilateral)
	Technique	Drug injected through a needle / catheter placed either directly into the spinal canal or immediately outside the spinal canal)epidural space).
	Risks	Possibility of headache, backache, buzzing in the ears, convulsions, infection, presistent weakness, numbness, residual pain, injury to blood vessels, arrhythmias.
☐ Major / minor nerve Block ☐ With Sedation ☐ Without Sedation	Expected Result	Temporary loss of feeling and / or movement of a specific limb or area.
	Technique	Drug injected near nerves providing loss of sensation to the area of the operation.
	Risks	Infection, swelling, presistent numbness, fits, residual pain, injury to blood vessels / lung injury/ collapse, allergic reaction.
☐ intravenous regional anaesthesia ☐ With Sedation ☐ Without Sedation	Expected Result	Temporary loss of feeling and / or movement of limb.
	Technique	Drug injected into vein of arm or leg while using a tourniquet
	Risks	Infection, fits, weakness, presistent numbness, residual pain, injury to blood vessels, allergic reaction, anaphylaxis, arrhythmias.
☐ Monitored Anaesthesia care (With sedation)	Expected Result	Reduced anxiety and pain, partial or total amnesia.
	Technique	Drug injucted into the bloodstream, breathed into the lungs, or by other routes producing a semi-conscious state.
	Risks	An unconscious state, depressed breathing, injury to blood vessels.

☐ Monitored Anaesthesia care (Without sedation)	Expected Result	Measurement of vital signs, availability of anaesthesia provide for further intervention.
	Risks	None
	Technique	Increased awareness, anxiety and / or discomfort

I / We hereby consent to the anaesthesia service checked above and autorize that it be administrated by doctor _____ or his or her all of whom are credentialed and privileged to provide anaesthesia service at Hospital. I also consent to an alternative type of anaesthesia, If necessary, as deemed appropriate by them.

I / We have also been explained about the post-operative pain management and other techniques for pain control.

I/ we understand that additional or specific risks may be involved due to certain associated systemic disease(s) identified in me / (my / our) patient.

The associated systemic disease(s) identified in me / (my / our) patient which / those responsible for high risk is/ are:

1._____

2._____

3._____

4._____

5._____

My signature (patient I authorized representative) below constitutes my acknowledgment that:

1. I have read, understood and agree tothe foregoingand it has been explained to me in the language understood by me.
2. All my questions have been satisfactorily answered and I have all the desired information to give an informed concent.
3. I hereby give my informed consent without any coercion or undue influence after understanding the proposed treatment, the risks, benefits, complications, possible alternative, the likelihood of success related to surgery/ procedure and possible problems related to recovery and possible results of non-treatment in my complete senses.

Name of Patient/patient's Relative	Relation with Patient	Sign of Patient/Patient's Relatives or Thumb Impression

Name of Consultant	Sign of Consultant

Witness (Staff Nurse)	Sign of Witness (Staff Nurse)

Date :_____ Time : _____ AM/PM

INTERPRETER'S STATEMENT

I have given translation in _____(state the patient's language here)of the consent form and assisted in the provision of any verbal and written information given to the patient/parent or guardian/ substitute decision-maker by the doctor.

Name of interpreter:_____ Signature: _____

Date:_____ Time:_____

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ADDRESSOGRAPH
If ADDRESSOGRAPH not Available, Write
PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

Pre Anaesthesia Evaluation (To be filled by Anaesthesiologist)

Procedure	Date : _____ Time : _____ AM/PM OT :
Anaesthesiologist:	Surgeon:
History from: Patient Relative Chart	Height: cm Weight : Kg BMI :
Chief Complaints/Surgical/Anaesthesia History	Medication

CVS	RS	CNS	Hepatic/Gt	Renal/Endocrine
☐ HTN_____ ☐ Valvular_____ ☐ CHF ☐ CHD_____ ☐ Arrythmia ☐ Pacemaker_____ ☐ IHD_____ ☐ METs_____	☐ URTI ☐ COPD/Asthma ☐ Bronchitis ☐ OSA ☐ Smoker_____	☐ CVA/TIA_____ ☐ Seizure_____ ☐ Neuro/Myopathy ☐ GCS _____	☐ Cirrhosis_____ ☐ Hepatitis ☐ GERD ☐ Alcohol _____	☐ DM _____ ☐ Thyroid_____ ☐ CDK _____ ☐ ARF_____

Examination HR _____ BP_____ RR_____ SPO2 _____ BHT _____

Investigations

CBC : Hb_____ TLC_____ Platelet _____

Renal : S.Urea_____ Creatinine_____ Na _____
K_____

LFT : Bilirubin___/___/___ SGPT _____ Albumin _____
PT/INR_____ apTT _____ RBS _____

Endocrine : TSH _____ T3 _____ T4 _____

Blood Group _____ HBsAg _____ HIV _____ HCV _____

ECG _____

2D ECHO _____

_____

Chest X-Ray/CT Scan/Sonography _____

_____

_____

Others _____

_____

ASA Grade

1☐ 2☐ 3☐ 4☐ 5☐ 6☐ E☐

Allergies

Airway Asessment
Dentition : normal/missing denture :
Mallampati Score :

Head & Neck / Spine

Dificult Intubation ☐

Risk Factors

Plan of Anaesthesia

Medication Orders :

Pre-Operative Orders

- NBM From _____
- Written Informed Consent
  ☐ Anaesthesia ☐ High risk
- Reserve _____ in blood bank
- FBS on _____
- Omit morning dose of insulin/OHA
- Investigations

Anaesthesiologist Name : Date : _____ Time : _____
Sign :

Immediate Pre-Anaesthesia Assessment

☐ Patient Identification ☐ NBM _____

Consent: ☐ Anaesthesia ☐ Surgery ☐ Significant Medical History _____

Pre Anaesthesia State ☐ Calm ☐ Apprehensive ☐ Awake ☐ Asleep ☐ Confused / Un Co-operative ☐ Unresponsive

Pulse _____ BP _____ RR _____ SpO2 _____ GCS _____

Anaesthesiologist Name:

Sign: Date Time:

Safety Checklist Date Time:

☐ Anaesthesia Equipment Checked ☐ Antibiotic Prophylaxis _____

Intra Operative Records

Procedure		OT	Start	End
		Anaesthesio		
Anesthesiologist	Surgeon	Surgery		

Anaesthesia Technique GA ☐ SA ☐ EA ☐ CSE ☐ NERVE BLOCK ☐ TIVA ☐ MAC ☐

General Anaesthesia

☐ Pre Oxygenation ☐ IV ☐ Inhalation ☐ RSI

☐ Pre Medication _____

_____

Induction Agent : _____

Muscle Relaxant : _____

Analgesia : _____

Monitoring

☐ ECG ☐ SpO2 ☐ EICO2 ☐ AGM

☐ NIBP ☐ Temp ☐ BIS ☐ NMM ☐ PNS

☐ CO Monitoring ☐ CVP _____ ☐ TEE

☐ ABP _____

☐ IV Cannula _____

☐ Naso/Orogastro Tube

☐ Foley's Catheter

General Anaesthesia

Laryngoscopy : ☐ Direct ☐ Fibre Optic ☐ Videolaryngoscope ☐ Blind ☐ Difficult ☐ Bougie / Stylet

Endotracheal Tube: ☐ Oral ☐ Nasal ☐ Cuff ☐ Uncuff ☐ DLT R/L ☐ Size _____ ☐ Fixed at _____

Airway: ☐ Oral ☐ Nasal ☐ LMA ☐ Igel ☐ Size _____

☐ Mask with Bain's Circuit ☐ Nasal Cannula ☐ Oxygen Mask ☐ Tracheostomy

Maintenance

O2 ☐ _____ % N2O ☐ Air ☐

☐ Controlled ☐ VCV / PCV ☐ Spontaneous ☐ SEVO

TV _____ RR _____ PEEP _____ ☐ DES

Position

Regional Anaesthesia

☐ Spinal ☐ Epidural ☐ CSE ☐ Nerve ☐ Block _____

☐ Sterile Preparation ☐ Local ☐ Nerve Locator ☐ USG Guidance

Position _____ Attempt _____ Local Space _____ Needle _____

Drug _____

Dose _____

_____

Sedation _____ Oxygenation _____

TIME															
MONITORING	220														
	200														
	NIBP v ABP ^ 180														
	160														
	⊙ HR 140														
	120														
	100														
	80														
	60														
	40														
VENTILATION	CVP PAP • 20														
	0														
	SpO_2														
	$ETCO_2$														
	T^0														
	ECG														
	EVENT														
	TV														
	PEEP														
	Ppeak														
Drugs															
Urine O/P															
IV Fluid															

Blood Transfusion ☐ PCV _____ ☐ FFP _____ ☐ PRC _____ ☐ SDP _____ ☐ Cryoprecipitats _____

TOURNIQUET	**Anticoagulation**		**Time**	**ACT**	**CPB**	ON
Applied at _____	Herparin					OFF
Released at _____					**X clamp**	ON
	Protamine					OFF

☐ Reversal Extubation : ☐ Done ☐ Not Done

Post Anaesthesia Care

Transferred to : ☐ Recovery ☐ ICU ☐ Ward Time : _____

☐ No O_2 ☐ O_2 Mask ☐ ETT/TT ☐ T piece ☐ Ventilator

Pulse _____ BP _____ RR _____ SpO_2 _____

Any Adverse Event

☐ Nausea/Vomiting	☐ Hypo/Hyperventilation	☐ Eye Injury
☐ Shivering	☐ Hypoxia	☐ Awareness during Anaesthesia
☐ Hypo/Hypertension	☐ Laryngospasm/Bronchospasm	☐ Pressure sore
☐ Arrhythmia	☐ Dental Injury	☐ Neurological complication

RECOVERY :

Recovery Start Time : _____ Recovery End Time : _____

Recovery Monitoring (To be done by recovery nurse)

Time	0 min	10 min	20 min	30 min	40 min	50 min	60 min	On discharge
Pulse								
BP Sys.								
BP Dia.								
SpO2								
RR								

Name of Nurse : _____ Sign with EID : _____ Time : _____

Recovery Sore : The patient should be shifted to ward when the score is ≥ 13

Score	Oxygenation	Respiration	Pulse	Circulation	Consciousness	Activity	Pain as per VAS	Associated Complaints
2	SpO2 > 92% on room air	Breathes deeply and coughs freely	62-90/min(Adult/ 10% of baseline (Paediatric)	BP+/-20 mmHg of normal (Adult)/ +/-10 mmHg of Baseline (Paed.)	Fully awake	Moves all extremities	1 to 5	Mild
1	SpO2 > 90% Oxygen	Dyspnoeic, shallow, or limited breathing	<60&>100/min (Adult)/15% of Baseline (Paediatric)	BP+/-20-50 mmHg of normal (Adult)/ +/-10-20 mmHg of Baseline (Paed.)	Arousable on calling	Moves two extremities	6 to 8	Moderate
0	SpO2 < 90% Oxygen	Apnoea	Controlled arrythmia with stable haemodynamic, with or without ST-T changes, with ECG changes	BP+/-50 mmHg of normal (Adult)/ >20 mmHg of Baseline (Paed.)	No response	No movement	9 to 10	Severe

Total Score

Post Operative Orders and Analgesia Plan :

_____

_____

_____

_____

_____

_____

_____

Name of Anaesthesiologist : _____ Sign : _____

Date : _____ Time : _____

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ADDRESSOGRAPH
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PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

CONSENT FORM FOR OPERATION / PROCEDURE

I _____ give my consent to

Dr. _____ for my relative

Mr./Ms._____for

Procedure / Operation.

Operation / Endoscopy Name: _____

(1) The operation / any other necessary procedures, important outcome, possible side effects (Complications) has been thoroughly explained to me by surgeon / staff.

(2) I have been explained that any examination / operation / endoscopy /'any other necessary procedure may not be completely safe and may be risky to any healthy person too.

(3) I give the permission to do any required changes or give medications as necessary.

(4) I have been informed that during or after the examination /operation / endoscopy / any procedure, there may be a possibility of any complication. e.g., cardiac arrest, severe haemmorrhage; infection, hypotension, pulmonary embolism etc. I trust fully that if it happens Surgeon Dr._____ or any other doctor appointed through them will treat me accordingly. I agree with their decisions. The extra cost of complication will be borne by me.
I have read this form/ this form has been read out to me.
I understand the above written instructions and I give the permission in fully conscious status; without any pressure.

(5) Risk : _____

_____

(6) Benefit : _____

_____

(7) Specific Complication :_____

_____

(8) PossibleAlternatives :_____

Date : _____ Name of the Patient :_____

Time : _____ Sign. of the Patient :_____

Consent taken by (Consultant doctor) :_____ Name of the Relative :_____

Name of the Unit Nurse :_____ Relation with Patient :_____

Name of Consultant doctor :_____ Sign. of the Relative :_____

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PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

ઓપરેશન / પ્રોસીજર માટે સંમતિ પત્ર

હું _____

ડૉ. _____ **ને મારા (સગા)**

_____ **નું ઓપરેશન / પ્રોસીજર / જરૂરી પ્રક્રિયા કરવાની સંમતિ આપું છું.**

ઓપરેશન/ પ્રોસીજર નું નામ: _____

(૧) આ તપાસ / ઓપરેશન / પ્રોસીજર / બીજી જરૂરી પ્રક્રિયાની અગત્યતા, પરિણામો અને આડઅસરો (કોમ્પલીકેશન) મને સર્જને / સ્ટાફે સમજાવેલ છે.

(૨) મને સમજાવેલ છે કે કોઈપણ તપાસ / ઓપરેશન / પ્રોસીજર / બીજી જરૂરી પ્રક્રિયા સંપૂર્ણ સુરક્ષિત નથી અને કોઈપણ સ્વસ્થ માણસ માટે પણ જોખમકારક થઈ શકે છે

(૩) હું એન્ડોસ્કોપીમાં અને ઓપરેશનમાં કોઈપણ જરૂરી પરિવર્તન કરવાની, જરૂરી દવાઓ આપવાની સંમતિ આપું છું.

(૪) મને જણાવવામાં આવ્યું છે કે તપાસ / ઓપરેશન / પ્રોસીજર / જરૂરી પ્રક્રિયા દરમ્યાન કે પછી થી ઈચ્છિત ફાયદાની જગ્યાએ આડઅસરો (કોમ્પલીકેશન) જેમકે હૃદય બંધ પડી જવું, અધિક રક્તસ્ત્રાવ, ચેપ, બી.પી.ઓછું થવું, પલ્મોનરી એમ્બોલીઝમ વગેરે થઈ શકે છે, અને હું વિશ્વાસ કરું છું કે આના માટે યોગ્ય ઉપાય સર્જન ડૉ..... અથવા તેમના દ્વારા નીમાયેલ કોઈપણ ડૉકટર દ્વારા કરવામાં આવશે તેના માટે હું સંપૂર્ણ સંમત છું અને કોમ્પલીકેશનની સારવાર માટે જરૂરી વધારાનો ખર્ચ આપવા માટે અમે તૈયાર છીએ.

(૫) જોખમ: .....

.....

(૬) ફાયદા: .....

.....

(૭) સંભવિત આડઅસર (નક્કી કરેલ ઓપરેશન માટે) : .....

.....

(૮) સંભવિત વિકલ્પ: .....

મેં, આ પત્ર (ફોર્મ) વાંચ્યું છે / આ ફોર્મ મને વાંચીને સંભળાવવામાં આવ્યું છે. હું ઉપર લખાયેલ ફોર્મ ને સમજું છું અને સંપૂર્ણ સભાન અવસ્થામાં અને કોઈપણ જાતના દબાણ સિવાય સંમતિ આપું છું.

તારીખ	:	પેશન્ટનું નામ	: .....
સમય	:	પેશન્ટની સહી	: .....
સંમતિ લેનારનું નામ (સર્જન)	:	સંબંધીનું નામ	: .....
યુનિટ નર્સનું નામ	:	દર્દી સાથેનો સંબંધ	: : .....
મેડિકલ ઓફિસરનું નામ	:	સંબંધીની સહી	: .....
સર્જનની સહી	:		

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CANCER ASSOCIATES
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AH/MRD/FORM - 55

ADDRESSOGRAPH
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SURGEON'S PREOPERATIVE NOTE

Patient Name :_____ Date:_____

Age/Sex :_____ Time:_____

L.D. No. :_____ Date :_____

Medical lliness :_____

_____

_____

NBM Hrs: _____

Name of Surgery : _____

_____

Special investigation :_____

_____

_____

Vitals :

Pulse	
SPO2	
RS	
CVS	
B.P.	
Others	

Patient Condition :_____

_____

_____

_____

Patient fit for Surgery : Yes ☐ No ☐

_____
Name & Sign surgeon

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OPERATIVE NOTES

Date:	Time:
Operating Surgeon:	Speciality:
Operating Surgeon:	Speciality:
Asst. Surgeon:	Speciality:
Anaesthetist:	
Scrub Nurse:	

Pre operative diagnosis :

Post operative diagnosis :

Surgery type : ☐ Clean ☐ Clean contaminated ☐ Dirty

Type of surgery	Name of operation
☐ Emergency	
☐ Elective	
Date:	Start Time: M, End Time: M, Total Hrs:
Type of anaesthesia :	☐ GA ☐ LA ☐ SPINAL ☐ EPIDURAL ☐ REGIONAL

OPERATIVE STEPS

Position
Identification of operating part
Incision
Operation findings

OPERATIVE STEPS

Important Steps

Drain :

Sponge & gauze count done :

Instrument count done :

Closure :

Skin closure with :

Sterile dressings kept.

INTRAOPERATIVE EVENTS / COMPLICATIONS :

Blood loss :	Blood Products used :
Implants used :	
Specimen send for HPE	
Immidiate post op. recovery	Patient transferred to : ☐ Room ☐ ICU care

POST OPERATIVE CARE SEE POST OPERATIVE ORDERS

Name of surgeon : _____

Sign : _____ Date / Time : _____

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POST OPERATIVE PAIN MANAGEMENT

Pain Management

Type : ☐ Single Shot ☐ Continuos ☐ Intermittent

Route : ☐ Block ☐ Infusion Pumps → IV Epidural

Drugs : Dilution _____
Rate _____

Date : _____

Time											
Pain Score (VAS)	0 No Pain	1 Just notice-able	2 Mild pain	3 Uncomfortable Pain	4 Annoying pain	5 Moderate pain	6 Just bearable	7 Strong pain	8 Severe pain	9 Horrible pain	10 Worst pain
Rescue Medication											
Vitals — Pulse											
Vitals — BP											
Vitals — SPO2											
Remarks											
Signature											

Date : _____

Time											
Pain Score (VAS)	0 No Pain	1 Just notice-able	2 Mild pain	3 Uncomfortable Pain	4 Annoying pain	5 Moderate pain	6 Just bearable	7 Strong pain	8 Severe pain	9 Horrible pain	10 Worst pain
Rescue Medication											
Vitals — Pulse											
Vitals — BP											
Vitals — SPO2											
Remarks											
Signature											

Continue...

Surgeon's Signature : _____

POST OPERATIVE ORDERS

Surgeon's Signature : _____

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AH/MRD/FORM - 54

PREOPERATIVE CHECKLIST

TO BE FILLED BY UNIT NURSE & OT NURSE

Date of Surgery : _____ **Time of Surgery :** _____ **Bed No. :** _____
Name of Surgeon : _____
Name of Anesthetist : _____ **Height :** _____ **cm**
Name of Surgery / Procedure : _____ **Weight :** _____ **kg**

High Risk : ☐ Yes ☐ No	HIV +ve : ☐ Yes ☐ No	HbsAg +ve : ☐ Yes ☐ No	Anti HCV +ve: ☐ Yes ☐ No

Allergy : _____ **Diagnosis :** _____

INVESTIGATIONS

	YES	No	No. of Copies
Biopsy Report	☐	☐	_____
Lab Reports	☐	☐	_____
X-Ray	☐	☐	_____
USG	☐	☐	_____
CT Scan	☐	☐	_____
MRI	☐	☐	_____
Doppler	☐	☐	_____

LATEST VITALS **Time :** _____

Temperature : _____ ^{0}F
Pulse : _____/ Min
Respiration : _____/ Min
B. P. : _____ mm of Hg
SpO2 : _____ %
RBS (If ordered) : _____ mg / dl

	YES	NO
1) Patient have ID Band with UHID No	☐	☐
2) Consent for Operation / Procedure taken	☐	☐
1) Informed Consent for surgery and Anesthesia		
2) High Risk (if applicable)		
3) Special Consent for Amputation		
4) Consent for Blood Product Transfusion		
3) OT Clearance done	☐	☐
4) Physician reference done	☐	☐
5) Preoperative anaesthetic check up done	☐	☐

	YES	NO
1) Any specific medication ordered by Surgeon / Physician /Anesthetist (if yes, Name it _____ .)	☐	☐
2) Any specific medication omitted by Surgeon / Physician /Anesthetist (if_yes Name it _____ .)	☐	☐
3) NBM from : _____ am / pm. Date :	☐	☐
4) Preparation of operating part by Betadine / Savlon done.	☐	☐
5) Shaving of operating part done.	☐	☐
6) Operative site is marked / mentioned right or left.	☐	☐
7) Prosthesis / Denture / Ornaments / Contact lens / Nail Polish / Lip stick Removed.	☐	☐
8) OT dress (Undergarments Removed)	☐	☐
9) Urine Voided / Bag Emptied with _____ ml urine (if catheterized)	☐	☐
10) Bowel Preparation done / Enema / Bowel wash / Laxative	☐	☐
11) Mouth care - gargles prescribed.	☐	☐
12) PCV required	☐	☐
13) PCV ready in Blood bank / in Hospital.	☐	☐

Unit Nurse : _____ **Sign with E.ID** _____ **Time :** _____
OT Nurse : _____ **Sign with E.ID** _____ **Time :** _____

SURGICAL / PROCEDURE SAFETY CHECK LIST

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CANCER ASSOCIATES
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Name of surgery: Date:

Before induction of anaesthesia Sign in (Time:_____)	Before skin incision / Onset of procedure (Time:_____) (Time Out:_____)	Before patient leaves the operating room / Cath lab Sign Out (Time:_____)
☐ Patient has confirmed Identity Site Procedure Consent	☐ Confirm all team members have introduced themselves by name and role	Nurse verbally confirms with the team: ☐ The name of the procedure recorded
☐ Site marked J Not applicable	☐ Surgeon, anaesthesiologist, perfusionist and nurse verbally confirm Patient Site Procedure	☐ The instrument, sponge, gauze and needle counts are correct (or not applicable)
☐ Anaesthesia safety check completed	Anticipated critical events ☐ Surgeon reviews: what are the critical or unexpected steps, operative duration, anticipated blood loss?	☐ How the specimen is labeled (Including patient name, specimen type, test and container)
☐ Pulse oximeter on patient & functioning	☐ Anaesthesia team reviews: Are there any patient-specific concerns?	☐ Whether there any equipment problems to be addressed
☐ Perfusion safety check completed	☐ Perfusion team reviews Are there any patient-specific concerns?	☐ Surgeon, anaesthesiologist and nurse review the key concerns for recovery and management of this patient
Does patient have a known allergy? ☐ No ☐ Yes	☐ Nursing team reviews: has sterility {Including indicator results) been confirmed? are there equipment issues or any concerns?	
Difficult airway / aspiration risk? ☐ No ☐ Yes, and equipment / assistance available.	Ready for all above	
Risk of > 500 ml blood loss (7 ml / kg In children)? ☐ No ☐ Yes, and adequate intravenous access and fluids planned	Has antibiotic prophylaxis been given within the last 60 minutes? ☐ Yes ☐ Not applicable Is essential imaging displayed? ☐ Yes ☐ Not applicable	
☐ I, as a transplant surgeon done double check for organ transplant, cross match of donor - recipient & ABO compatibility.		

Consultant	Anaesthetist	Perfusionist /Cath technician	Scrub Nurse	OT / Cath Incharge

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PATIENT & FAMILY EDUCATION FORM
(પેશન્ટ અને ફેમિલી એજ્યુકેશન ફોર્મ)

Diagnosis: **નિદાન:**
Primary Consultant: **પ્રાથમિક તબીબ:**

Assessment of Barrier to Learning (To be filled by Medical Officer) :
શીખવામાં અવરોધનું મૂલ્યાંકન (મેડિકલ ઓફિસર દ્વારા ભરવામાં આવશે) :

Language (ભાષા) :	**Cognition (જ્ઞાન) :**	**Level of Consciousness :** **સભાનતાનું સ્તર :**
Hearing :	**Vision :**	**Family or Religious beliefs :**
સાંભળવાનું :	**દ્રષ્ટિ :**	**કૌટુંબિક અથવા ધાર્મિક માન્યતાઓ :**
Others (બીજું કંઈ) :		

Patient Education Details (દર્દીના શિક્ષણની વિગતો) :

Signature of concern person		**Patient & Family Education Information Topics (દર્દી અને કુટુંબ શિક્ષણ માહિતીના વિષયો)**				
	Patient Rights & Responsibilities awareness (By floor manager) દર્દીના અધિકારો અને જવાબદારીઓ અંગે જાગૃતિ (ફ્લોર મેનેજર દ્વારા)	Safe & Effective use of medication /potential side effect (By Medical Officer) દવાઓનો સલામત અને અસરકારક ઉપયોગ/ સંભવિત આડઅસર (મેડિકલ ઓફિસર દ્વારા)	Food - Drug Interaction (By Dietitian) ખોરાક - દવાની ક્રિયાપ્રતિક્રિયા (આહારશાસ્ત્રી દ્વારા)	Pain Management (By Medical Officer) પીડા વ્યવસ્થાપન (મેડીકલ ઓફિસર દ્વારા)	Patient Falls (By Nursing Staff) દર્દીનું પડી જવું (સ્ટાફ નર્સ દ્વારા)	Hand Hygiene (By ICN) હાથની સ્વચ્છતા (આઈ.સી.એન. દ્વારા)
Clinical staff name and signature						
Patient/Patient relative name and signature						
Clinical staff name and signature						
Patient/Patient relative name and signature						

Please make entry with Sign, Name, Date & Time (કૃપા કરીને સહી, નામ, તારીખ અને સમય સાથે એન્ટ્રી કરો)

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AH/MRD/FORM - 45

PATIENT & FAMILY COUNSELING

Explained by	Date & Time	Information Given	To Whom	How	Response	Sign of Relative	Sign of H.C.W.

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AH/MRD/FORM - 152

V.I.P. SCORE (Visual Infusion Phlebitis Score)

Observation	Score		Symptoms	Action
IV Set Appear Healthy	0	>	No sign of phlebitis	Observe Cannula
One of the following is evident Slight pain near IVor Slight redness near IV	1	>	Possible first sign	Observe Cannula
Two of the following are evident Pain at IV site Erythema or Swelling	2	>	Early stage of phlebitis	Re-site Cannula
All the following signs are evident Pain along path of cannula Erythema or induration	3	>	Mid-stage of phlebitis	Re-site Cannula and consider treatment
All the following signs are evident and extensive 1. Pain along path of cannula 2. Erythema 3.Induration 4.Palpable venous cord	4	>	Advance stage of phlebitis or start of thrombophlebitis	Re-site Cannula and considertreatment
All the following signs are evident and extensive1. Pain along path of cannula 2. Erythema 3.Induration 4.Palpable venous cord 5. Pyrexia	5	>	Advance stage of thrombophlebitis	Initiate Treatment

DATE	MORNING			EVENING			NIGHT			Reason for Cannula change	Size & Site of Insertion (If Change)	REMARK
	SCORE		Emp Id Staff	SCORE		Emp Id Staff	SCORE		Emp Id Staff			

DVT SCORING

Criteria	Score	Criteria	Score
Age> 65 years	1	Hemoptysis	2
History of DVT or Pulmonary Embolism History	3	Heartrate 75-95/min	3
Surgery or lower limb fracture≤ 1 month	2	Heartrate > 95/min	5
Activemalignant condition	2	Pain on deep palpation of lower limb and unilateral edema	4
Unilaterallowerlimbpain	3	**TOTAL SCORE**	

DVT Intervention

Sr.no	Date	DVT Score	Limb Elevation	Stockings	Medication	Staff sign	Remarks

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DVT (Risk Factor Assessment) (to be filled by doctor)

Diagnosis: _____

Each Risk Factor Represents 1 Point (A)

- ☐ Age 41-60 years
- ☐ Minor surgery
- ☐ Pregnancy or postpartum (<1 month)
- ☐ Varicose veins
- ☐ Inflammatory bowel disease
- ☐ Obesity (BMI >30)
- ☐ Oral contraceptives or hormone replacement therapy

TOTAL SCORE (A) : _____

Each Risk Factor Represents 2 Points (B)

- ☐ Age 60-74 years
- ☐ Major surgery (> 60 minutes)
- ☐ Immobilizing Plaster Cast
- ☐ Malignancy
- ☐ Central venous access
- ☐ Medical or Surgical Patients confined to bed >72 hours

TOTAL SCORE (B) : _____

Each Risk Factor Represents 3 Points(C)

- ☐ History of DVT/PE
- ☐ Acute myocardial infarction
- ☐ Congestive heart failure
- ☐ Sepsis
- ☐ Positive Factor V Leiden
- ☐ Positive Prothrombin 20210A
- ☐ Elevated serum homocysteine
- ☐ Positive Lupus anticoagulant
- ☐ Protein C and S
- ☐ Elevated anticardiolipin antibodies
- ☐ Heparin-induced tluombocylopenia (HIT)
- ☐ Disorders of plasminogen and plasmin activation
- ☐ Antithrobmin 3 deficiency

TOTAL SCORE (C) : _____

Each Risk Factor Represents 5 Points(D)

- ☐ Elective major lower extremity arthroplasty
- ☐ Hip, pelvis or leg fracture
- ☐ Stroke
- ☐ Multiple trauma
- ☐ Acute spinal cord injury

TOTAL SCORE (D) : _____

Total Score (A+B+C+D) ☐

DVT Risk and Suggested Prophylaxis For Surgical Patients

Total Risk Factor Score	Risk Level	Prophylaxis Regimen
0-1	Low Risk	No specific measures; early ambulation
2	Moderate Risk	ES, IPC, LDUH (5000U BID),or LWMH (<3400 U)
3-4	High Risk	IPC, LDUH (5000U TID), or LMWH (>3400U)
5 or more	Highest Risk	Pharmacological: LDUH, LMWH (>3400 U)*, Warfarin*, or FXa I* alone or in combination with ES or IPC

ES - Elastic Stockings **IPC -** Intermittent Pneumatic Compression

LDUH - Low Dose Unfractionated Heparin
LMWH-Low Molecular Weight Heparin

Fxa I - Factor X Inhibitor

Prophylaxis: _____

_____

_____

Name of Doctor: _____ **Signature:** _____

Date &Time: _____

MEWS (Modified Early Warning System)

	3	2	1	0	1	2	3
Respiratory Rate per minute		Lass Then 8		9-14	15-20	21-29	More Then 30
Heart Rate per minute		Lass Then 40	40-50	51-100	101-110	111-129	More Then 129
Systolic Blood Pressure	Lass Then 70	71-80	81-100	101-199		More Then 200	
Conscious level (AVPU)	**U**nersponsive	Lass Then 35.0	Responds to **V**oice	**A**lert	New agitation Confusion		
Temperature (°c)		Lass Then 70	36.1-36	36.1-38	38.1-38.5	More Then 28.6	
Hourly Urine For 2 Hours	Lass Then 10mls / hr	Lass Then 30mls / hr	Lass Then 45mls / hr				

EARLY WARNING SCORING SYSTEM FOR DETECTING ADULT PATIENTS WHO HAVE OR ARE DEVELOPING CRITICAL ILLNESS

IS THE SCORE FOR YOUR PATIENT 1-2? PERFORM 2 HOURLY OBSERVATIONS AND INFORM NURSE IN CHARGE

IS THE SCORE FOR YOUR PATIENT 3? PERFORM 1-2 HOURLY OBSERVATIONS AND INFORM NURSE IN CHARGE

*IF THE MEWS SCORE IS DETERIORATING : THE WARD S.H.O. OR DUTY DOCTOR **MUST** ATTEND*

IS THE SCORE FOR YOUR PATIENT 4 OR MORE? PERFORM OBSERVATIONS AT LEAST 1/2 HOURLY. ENSURE MEDICAL ADVICE IS SOUGHT AND CONTACT OUTREACH TEAM (see below)

SHUBHAM CANCER CENTRE

Near Sola Over Bridge, Hebatpur Road,
Opp. Zydus Hospital, Thaltej,
Ahmedabad 380 059, (Gujarat).
Mobile No.: +91 95123 40431

CANCER ASSOCIATES
CONSULTANT CANCER SURGEONS

ADDRESSOGRAPH

If ADDRESSOGRAPH not Available, Write
PT. Name, IP No., UHID, Age, Sex, Date, Consultant Name

Date											
Time											
Name Of Staff											
Sign											
Respiratory Rate per Min.											
Heart Rate per Min.											
Systolic B.P											
Conscious Lvel - AVPU											
Temp - *C											
Hourly Urine For 2 Hours											
Total Score											
Remarks											

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AH/MRD/FORM - 155 Page no : 1

HAI BUNDLE CHECK LIST

CAUTI BUNDLE

Sr. No.	Elements of bundle care	DAY 1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	25	26	27	28	29	30	31
1	Hand Washing Before & after																															
2	Avoid unnecessary urinary catheterization																															
3	Insert catheter using antiseptic technique																															
4	Daily Catheter care Review																															
5	Empty the urine bag when % filled																															
6	Secure Catheter at Mid-thigh to prevent movement																															
7	Clean perineal are with normal saline, soad & water																															
8	Keep the urinary bag below level of Bladder to maintain the unabstructed urine flow																															
	Name of Staff																															
	Verified By ICN																															

SSI BUNDLE

Sr. No.	Elements of bundle care	DAY 1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	25	26	27	28	29	30	31
1	Hand Washing Before & after patient care																															
2	Daly Dressing'surgical area assessment																															
3	Use sterile technique																															
4	Educate patient relative about symptoms of SSI																															
5	Review the site during dressing																															
	Name of Staff																															
	Verified By ICN																															

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AH/MRD/FORM - 50

INPATIENT FEEDBACK

Name of the Patient:		**UHID No:**	

તમે તમારા મૂલ્યવાન સમયને તમારા સાથી/મિત્રના ઉપચારના સંબંધમાં હોસ્પિટલમાં વિતાવ્યા છે. તમને વિનંતી કરવામાં આવે છે કે તમે જે દર્દીઓની મુલાકાત લીધી છે, તેઓની ગુણવત્તા વિશે તમારા અભિપ્રાય જણાવો. તમારા દ્વારા પ્રદાન કરવામાં આવેલી માહિતી ગોપનીય રાખવામાં આવશે અને સેવાઓનો ઉપયોગ કરવામાં ફક્ત તેનો ઉપયોગ કરવામાં આવશે.

મહેરબાની કરીને યોગ્ય બૉક્સ પર ટીક કરો અને સૂચન બોક્સમાં પ્રશ્નાવલિ છોડો.

ક્રમાંક નં.	માપદંડ	ખરાબ ૧	ઠીક ૨	સારી ૩	ખુબ સારી ૪	ઉત્તમ ૫	કોઈ ટીપ્પણીઓ નહિ
1.	હોસ્પિટલમાં પૂરતી માહિતી ઉપલબ્ધ (દિશાસૂચક અને સ્થાન સંકેતો, નોંધણી કાઉન્ટર, દવા, વગેરે)						
2.	નોંધણી કાઉન્ટર પર રાહ જોવી	૩૦ મિનિટથી વધારે	૧૦ થી ૩૦ મિનિટ	૫ થી ૧૦ મિનિટ	૫ મિનિટની અંદર	તરત જ	
3.	હોસ્પિટલમાં કર્મચારીઓની વર્તણૂક અને વલણ નોંધણી/પ્રવેશ કાઉન્ટર પર						
4.	ડિસ્ચાર્જ પ્રક્રિયા પર તમારી પ્રતિક્રિયા						
5.	વૉર્ડની સ્વચ્છતા						
6.	શૌચાલયની સ્વચ્છતા						
7.	બેડ શીટ્સ, ઓશીકું, વગેરેની સ્વચ્છતા						
8.	આસપાસના અને કેમ્પસ ડ્રેઇનની સ્વચ્છતા						
9.	ડૉકટરના ધ્યાનની નિયમિતતા						
10.	ડોકટર્સનું વલણ અને સંચાર						
11.	દર્દી અને પરામર્શની તપાસ માટે સમય પસાર કર્યો						
12.	વૉર્ડમાં નર્સ દ્વારા આપવામાં આવતો શીઘ્ર પ્રતિભાવ						
13.	વૉર્ડમાં નર્સની ચોવીસ કલાક ઉપલબ્ધતા						
14.	નર્સની વલણ અને સંચાર						
15.	વોર્ડબોય/આયાબેનની ઉપલબ્ધતા, વલણ અને શીઘ્રતા						
16.	તમામ સૂચિત દવાઓ હોસ્પિટલમાંથી ઉપલબ્ધ કરાઈ હતી						
17.	ડૉકટરના જ્ઞાન વિષેની તમારી માન્યતા						
18.	હોસ્પિટલમાં ડાયગ્નોસ્ટિક્સ સેવાઓ પ્રદાન કરવામાં આવી હતી						
19.	આપવામાં આવેલ આહારની ગુણવત્તા						
20.	સારવાર દરમિયાન તમારો સંપૂર્ણ સંતોષ						

૧. તમે હોસ્પિટલમાં શું સુધારણા જોવા માંગો છો ?

૨. તમે સારવાર માટે આ હોસ્પિટલમાં કેમ આવ્યા છો ?

૩. શું તમે સારવાર માટે આગલી વખતે આ હોસ્પિટલમાં પાછા આવવા માંગો છો ?

૪. તમારા મૂલ્યવાન સૂચનો

૫. તમે શુભમ કેન્સર સેન્ટર (કેન્સર એસોસિએટ્સ) ની ભલામણ કોઈ મિત્ર અથવા સહકર્મીને કરો છો?

Not at all likely | 0 | 1 | 2 | 3 | 4 | 5 | 6 | 7 | 8 | 9 | 10 | Extremely likely

Date _____ **Ward** _____ **Age** _____ **Sex** _____ **Date of Admission** _____

Time_____ **Sign** _____

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AH/MRD/FORM - 50

INPATIENT FEEDBACK

Name of the Patient:		**UHID No:**	

Dear Client,

You have spent your valuable time in the hospital in connection with your / relative's/ friend's treatment. You are requested to share your opinion about the quality of services, which you experienced, while staying in the hospital. The information provided by you would be kept confidential, and would only be used for improving the services.

Please tick the appropriate box and drop the questionnaire in the Suggestion box

SR No	Attributes	Poor 1	Fair 2	Good 3	Very Good 4	Excell -ent 5	No comments
1.	Availability of sufficient information at Registration/Admission counter (Directional & location signages, Registration counter, Laboratory, Radiology Department, Dispensary, etc)						
2.	Waiting time at the Registration/Admission counter	more than 30 Mins	10-30 Mins	5-10 Mins	Within 5 Mins	Immedia te	
3.	Behaviour and attitude of hospital staff at the registration/ admission counter						
4.	Your feedback on discharge process						
5.	Cleanliness of the ward						
6.	Cleanliness of Bathrooms & toilets						
7.	Cleanliness of Bed sheets, pillow-covers, etc						
8.	Cleanliness of surroundings and campus drains						
9.	Regularity of Doctor's attention						
10.	Attitude & communication of Doctors						
11.	Time spent for examination of patient and counselling						
12.	Promptness in response by Nurses in the ward						
13.	Round the clock availability of Nurses in the ward						
14.	Attitude and communication of Nurses						
15.	Availability, attitude & promptness of Ward boys/girls						
16.	All prescribed drugs were made available from Hospital Supply						
17.	Your Perception of Doctor's knowledge						
18.	Diagnostics Services were provided within the hospital						
19.	Timeliness of supply of the diet and its quality						
20.	Your overall satisfaction during the treatment as an in - patient						

1. What improvement would you like to see in the hospital
2. What made you come to this hospital for treatment?
3. Would you like to return to this hospital next time for treatment
4. Your valuable suggestions
5. How likely are you to recommend Shubham Cancer Centre (Cancer Associates) to a friend or colleague ?

Not at all likely | 0 | 1 | 2 | 3 | 4 | 5 | 6 | 7 | 8 | 9 | 10 | Extremely likely

Date _____ Ward _____ Age _____ Sex _____ Date of Admission _____

Time_____ Sign _____